STAFF INFO
Name: Lara, Toni RN
Patient INFO
Name: WILLIAMS, MARY
DOB: 1947-04-27

OASIS Assessment: OASIS E1 - START OF CARE

ADMINISTRATIVE INFORMATION

(M0010) Agency Medicare Provider Number
037834

(M0014) Branch State
Branch State: **Arizona**

(M0016) Branch ID Number

(M0018) Attending Physician (NPI)
1356344824

Physician:
STEVENS, WILLIAM

Address:
3033 N 44TH ST STE 100 PHOENIX AZ 85018

☐ **Not Applicable**

(M0020) Patient ID number
82

(M0030) Start of Care Date
02/26/2026

(M0032) Resumption of Care Date
mm/dd/yyyy

☑ **Not Applicable**

(M0040) Patient Name

First Name:	Middle Name:	Last Name:	Suffix:
MARY		WILLIAMS	

(M0050) Patient State of Residence
State of Residence: **Arizona**

(M0060) Patient Zip Code
85086

(M0063) Medicare Number
7MU9RM0FW51

☐ **Not Applicable**

(M0065) Medicaid Number

☑ **Not Applicable**

(M0064) Social Security Number

☑ **Not Applicable**

(M0066) Birth Date *
04/27/1947

(M0069) Gender *
Gender: **Female**

(M0080) Discipline Completing Assessment:
● 1. RN ○ 2. PT ○ 3. SLP/ST ○ 4. OT

Assigned Staff:
Lara, Toni RN

(M0090) Date Assessment Completed:
02/26/2026

(M0100) Assessment Reason
● 1. Start of Care

(A1005) Ethnicity Are you of Hispanic, Latino/a, or Spanish origin? Check all that apply

- ☑ A. No, not of Hispanic, Latino/a, or Spanish origin
- ☐ B. Yes, Mexican, Mexican American, Chicano/a
- ☐ C. Yes, Puerto Rican
- ☐ D. Yes, Cuban
- ☐ E. Yes, Another Hispanic, Latino, or Spanish origin
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond

(A1010) Race What is your race?

- ☑ A. White
- ☐ B. Black or African American
- ☐ C. American Indian or Alaska Native
- ☐ D. Asian Indian
- ☐ E. Chinese
- ☐ F. Filipino
- ☐ G. Japanese
- ☐ H. Korean
- ☐ I. Vietnamese
- ☐ J. Other Asian
- ☐ K. Native Hawaiian
- ☐ L. Guamanian or Chamorro
- ☐ M. Samoan
- ☐ N. Other Pacific Islander
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond
- ☐ Z. None of the above

(A1110) Language

A. What is your preferred language?
English

B. Do you need or want an interpreter to communicate with a doctor or health care staff?
● 0 - No ○ 1 - Yes ○ 9 - Unable to determine

(M0102) Date of Physician-ordered Start of Care (Resumption of Care)
If the physician indicated a specific start of care (resumption of care) date when the patient was referred for home health services, record the date specified.
Skip to A1250, Transportation, if date entered:
02/26/2026

☐ **NA - No specific SOC/ROC date ordered by physician**

(M0104) Date of Referral:
Indicate the date that the written or verbal referral for initiation or resumption of care was received by the HHA:
mm/dd/yyyy

(M0150) Current Payment Sources for Home Care: (Mark all that apply.) *

- [ ] 0. None; no charge for current services
- [x] 1. Medicare (Traditional fee-for-service)
- [ ] 2. Medicare (HMO/managed care/Advantage plan)
- [ ] 3. Medicaid (Traditional fee-for-service)
- [ ] 4. Medicaid (HMO/managed care)
- [ ] 5. Worker's compensation
- [ ] 6. Title programs (for example, Title III, V, or XX)
- [ ] 7. Other government (for example, TriCare, VA)
- [ ] 8. Private insurance
- [ ] 9. Private HMO/managed care
- [ ] 10. Self-pay
- [ ] 11. Other (Specify):
- [ ] UK - Unknown

(A1250) Transportation Has lack of transportation kept you from medical appointments, meetings, work, or from getting things needed for daily living?

- [x] A. Yes, it has kept me from medical appointments or from getting my medications
- [ ] B. Yes, it has kept me from non-medical meetings, appointments, work, or from getting things that I need
- [ ] C. No
- [ ] X. Patient unable to respond
- [ ] Y. Patient declines to respond

(M1000) Inpatient Facilities that patient was discharged within the past 14 days? (Mark all that apply.)

- [ ] 1. Long-term nursing facility (NF)
- [ ] 2. Skilled nursing facility (SNF/TCU)
- [ ] 3. Short-stay acute hospital (IPPS)
- [ ] 4. Long-term care hospital (LTCH)
- [ ] 5. Inpatient rehabilitation hospital or unit (IRF)
- [ ] 6. Psychiatric hospital or unit
- [ ] 7. Other (specify):
- [x] NA - Patient was not discharged from an inpatient facility _ **[Go to M1021]**

(M1005) Inpatient Discharge Date (most recent):

mm/dd/yyyy

- [ ] UK - Unknown

Primary Reason for Home Health/Medical Necessity:

Mary Williams is a 78-year-old female with a significant functional limitation due to progressive spinal deformity, resulting in severe lower back pain that restricts her activities of daily living (ADLs). She has a medical history that includes arthritis, atrial fibrillation, chronic pain, type 2 diabetes mellitus, osteoarthritis, osteopenia, and a history of multiple hospitalizations due to pneumonia. The patient also experiences significant psychosocial challenges, including a lack of assistance at home, as her husband does not help with household tasks, further compounding her difficulties in managing her health and home environment. Mary's current medical status is concerning, as she demonstrates severe sagittal imbalance and coronal shift, which not only exacerbates her back pain but also impairs her mobility and overall functionality. She has reported an inability to stand for long periods and struggles with basic tasks such as folding laundry, leading to increased frustration and emotional distress. Her recent hospitalizations and ongoing symptoms highlight the necessity for continuous skilled care, particularly in pain management and rehabilitation.

Given her complex clinical picture, home health care is medically necessary to provide skilled nursing interventions and physical therapy aimed at pain management, mobility enhancement, and support with ADLs. The care team will focus on optimizing her pain control through medication management and exploring potential interventional pain management options, as well as ensuring that she receives necessary support for her daily activities. Additionally, the team will assess her home safety and make recommendations for adaptive equipment or home modifications to enhance her functional capacity and quality of life.

In summary, Mary's admission to home health is justified due to her severe spinal deformity and associated functional limitations, the need for skilled nursing and therapy services, and her recent hospitalizations that indicate a significant medical need for support in her recovery and chronic illness management. This approach aligns with Medicare home health guidelines, ensuring that she receives the comprehensive care required to improve her health outcomes.

Homebound Reason

- [ ] Document as Clinical Narrative
- [ ] Patient is not Homebound

Criteria One (One must be met): *Describe what assistive devices the patient is dependent on (if any) and specifically why the patient condition warrants the special transportation or the assistance of another person or describe the condition that makes leaving home medically contraindicated.*

Criteria 1 A: Because of illness or injury

- [x] Patient needs the aid of supportive devices
  - [ ] Crutches
  - [x] Cane
  - [ ] Manual Wheel Chair
  - [x] Walker
  - [ ] Motorized Wheelchair
  - [ ] Scooter
- [ ] Patient Needs the use of special transportation.
- [x] Patient needs the assistance of another person to leave the place of residence

Criteria 1 B: Has a condition such as leaving his or her home is medically contraindicated.

- [ ] Status post surgery
- [ ] Angina upon minimal exertion
- [ ] Immuno deficiency

Other Homebound Reason :

Patient is experiencing chronic pain and has deforming dorsopathies (unspecified site).

Criteria Two (Both must be met)

Criteria 2 A: There must be exist a normal inability to leave home.

- [ ] Dyspnea on minimal exertion
- [ ] Bedbound
- [ ] Chair bound
- [ ] Dementia
- [ ] Blindness
- [ ] Angina upon minimal exertion
- [ ] Paralysis
  - [ ] Quadprilegic or
  - [ ] Paraplegic or
  - [ ] Hemiplegia
- [x] Unable to tolerate standing or walking for extended periods
- [ ] Weakness and pain from recent surgery
- [ ] Experiencing pain impairs/restricts mobility
- [ ] Visually impaired-requires constant attendance/assist

- [x] Balance impaired with potential for falls; uses walls and furniture to ambulate
- [x] Leaving home is exhausting, extended rest periods to recover
- [ ] Extended respiratory recovery after exertion
- [ ] Requires human assist due to unsteady gait/poor balance
- [ ] Cognitively impaired, unable to safely leave home without assist
- [ ] Infectious disease process causing weakness and exhaustion
- [ ] Narcotics impaired ability to safely leave home unassisted
- [ ] Bedbound and requires EMS support

Other Homebound Reason :

Leaving home increases the risk of complications, such as delayed healing, infection, and further tissue damage.

Criteria 2 B: Leaving home must require a considerable and taxing effort

- [ ] Special transportation
- [ ] Severe pain on ambulation
- [ ] Leaving familiar environment causes fear and confusion
- [x] Leaving home is strenous requiring extended rest to recover
- [ ] Fatigue to arrange/use appropriate transportation
- [x] Requires assistive device to ambulate
- [ ] Exhausting to navigate vehicle and require human assist
- [ ] Requires assist to manage equipment and public transportation
- [ ] Needs assistance to handle and transport equipment
- [ ] Activities restricted due to pain limitations

Other Homebound Reason :

Mobility outside the home could potentially exacerbate the condition, hinder recovery and increase the likelihood of additional health issues.

Prognosis

- ( ) 1-Poor
- ( ) 2-Guarded
- (x) 3-Fair
- ( ) 4-Good
- ( ) 5-Excellent

Functional Limitations

- ( ) 1 - Amputation
- ( ) 2 - Bowel/Bladder (Incontinence)
- ( ) 3 - Contracture
- ( ) B- Other (Specify)
- ( ) 4 - Hearing
- ( ) 5 - Paralysis
- (x) 6 - Endurance
- (x) 7 - Ambulation
- ( ) 8 - Speech
- ( ) 9 - Legally blind
- ( ) A - Dyspnea with minimal exertion

Emergency Preparedness:

Discussed the HHA's plans for supporting their patients during a natural or man-made disaster
Obtained the patient's emergency contact numbers for the medical record
Discussed the development of the patient's individualized emergency preparedness plan of care, including self-care readiness and the procedure to follow up with the HHA in the event services are interrupted
Discussed patient specific emergency planning options
Educational materials provided to suggest/assist with emergency management/decision making priorities
Written materials to restate/reinforce the emergency preparedness procedures given to the Patient, and Representative.
List of recommended items to have prepared/ready and available in the event of an emergency

Acuity Level:
Medium Risk or Medium Priority

Emergency Triage:
Category 2 - Not life threatening but would suffer severe adverse effect

Additional Emergency Preparedness Information

- [x] Needs assistance during an emergency
- [x] Contact made with local/state emergency preparedness officials regarding patient in need of help during an evacuation
- [x] Medical Needs/Equipment (i.e. bedbound, oxygen, vent, IV cardiac meds other DME)

Comments:

Advance Directives

Patient has Advance Directives? ( ) Yes (x) No

If you have checked "yes", fill the following fields:

- [ ] Healthcare Representative
- [ ] Do Not Resuscitate
- [ ] Power of Attorney
- [ ] Living Will
- [ ] Conservator Assigned
- [ ] Organ Donor
- [ ] Funeral Arrangements Made
- [x] Full Code — Copies to Follow
- [ ] The patient has not executed any advance directives to health care.
- [ ] The patient was provided with written and verbal information on advance directives.

Comments:

Has surrogate? ( ) Yes (x) No

Surrogate Name:

Surrogate Phone:

Cultural

Primary Language?
- [x] English
- [ ] Spanish
- [ ] Chinese
- [ ] Vietnamese

Other:

Is religion important to the patient? (x) Yes ( ) No

Patient's religious preference:
Christian

Patient's primary source of emotional support:
Son

Past Medical History (Mark all that apply)

- [ ] CHF
- [x] Chest Pain
- [ ] Cardiomyopathy
- [ ] MI
- [x] Arrhythmia
- [ ] CAD

- [x] HTN
- [ ] Cancer:
- [ ] PVD
(Specify Type):
- [ ] Murmur
In remission?
( ) Yes
(x) No
- [ ] Osteoathritis/DJD:
(Specify site affected):
- [ ] Joint Replacement:
(Specify joint):
- [ ] Rheumatoid Arthritis
- [x] Gait Problems
- [x] Fractures
- [x] Falls

- [ ] CVA
- [ ] TIA
- [ ] MS
- [ ] Hemiplegia
- [ ] Seizures
- [ ] Headaches
- [ ] Dizziness/Vertigo

- [ ] IBS
- [ ] Crohn's Disease
- [ ] Diverticulitis/Diverticulosis
- [ ] Constipation
- [x] Diarrhea
- [ ] Fecal Incontinence
- [ ] Liver/Gallbladder Problems:
(Specify):

- [x] Depression
- [ ] Anxiety
- [ ] Dementia
- [ ] Substance Abuse:
(Specify):
- [ ] Alzheimer's
- [ ] Mental Disorder:
(Specify):

- [ ] Pressure Ulcer
- [ ] Stasis Ulcer
- [ ] Diabetic Ulcer
- [ ] Trauma Wound
- [ ] Other: (Specify):
- [ ] Chronic Kidney Disease
- [ ] Renal Failure
- [ ] Dialysis

- [ ] Anemia
- [ ] Abnormal Coagulation
- [ ] Blood Clots
- [ ] Diabetes
- [x] Thyroid Problems

- [ ] COPD
- [ ] Asthma
- [ ] Emphysema
- [ ] Chronic Obstructive Bronchitis
- [ ] Chronic Obstructive Asthma

- [ ] Urinary Incontinence
- [ ] Urinary Retention
- [ ] BPH
- [ ] Recent/Frequent UTI
- [ ] Tuberculosis
- [ ] Hepatitis:
(Specify):
- [ ] Infectious Disease:
(Specify):

- [ ] Tobacco Dependence

Type: Amount: Length of Time Used:

Past Surgical History:
Appendectomy back surgery times 4 had titanium rods placed, total hysterectomy

Health Screening

Last Cholesterol Level: mm/dd/yyyy
Last Mammogram: mm/dd/yyyy

Does patient perform monthly self breast exam? ( ) Yes (x) No

Last Pap Smear: mm/dd/yyyy
Last PSA: mm/dd/yyyy

Last Prostate Exam: mm/dd/yyyy
Last Colonoscopy: mm/dd/yyyy

(M1100) Patient Living Situation. Which of the following best describes the patient's residential circumstance and availability of assistance?

Living Arrangement	Around the clock	Regular daytime	Regular Night time	Short-term Assistance	No Assistance available
A. Patient lives alone	( ) Patient Lives alone around the clock	( ) Patient Lives alone Regular daytime	( ) Patient Lives alone Regular Night time	( ) Patient Lives alone Short-term Assistance	( ) Patient Lives alone No Assistance available
B. Patient lives with other person(s) in the home	(x) Patient lives with other person(s) in the home Around the clock	( ) Patient lives with other person(s) in the home Regular daytime	( ) Patient lives with other person(s) in the home Regular Night time	( ) Patient lives with other person(s) in the home Short-term Assistance	( ) Patient lives with other person(s) in the home No Assistance available
C. Patient lives in congregate situation (for example, assisted living, residential care home)	( ) Patient lives in congregate situation (for example, assisted living, residential care home) around the clock	( ) Regular daytime	( ) Patient lives in congregate situation Regular Night time	( ) Patient lives in congregate situation Short-term Assistance	( ) Patient lives in congregate situation No Assistance available

Caregiver Contact Info:

Primary Caregiver: Darren Williams
Relationship to Patient: Spouse
Contact Number: 480-276-1795

Comments:

Name of Facility:

Phone:

Able to safely care for patient? (x) Yes ( ) No

ACTIVE DIAGNOSES

(M1021/1023) Diagnoses and Symptom Control

List each diagnosis for which the patient is receiving home care in Column 1, and enter its ICD-10-CM code at the level of highest specificity in Column 2 (diagnosis codes only – no surgical or procedure codes allowed). Diagnoses are listed in the order that best reflects the seriousness of each condition and supports the disciplines and services provided. Rate the degree of symptom control for each condition in Column 2. ICD-10-CM sequencing requirements must be followed if multiple coding is indicated for any diagnoses.

Code each row according to the following directions for each column:

- **Column** 1: Enter the description of the diagnosis. Sequencing of diagnoses should reflect the seriousness of each condition and support the disciplines and services provided. Column 2: Enter the ICD-10-CM code for the condition described in Column 1 – no surgical or procedure codes allowed. Codes must be entered at the level of highest specificity and ICD-10-CM coding rules and sequencing requirements must be followed. Note that external cause codes (ICD-10-CM codes beginning with V, W, X, or Y) may not be reported in M1021 (Primary Diagnosis) but may be reported in M1023 (Secondary Diagnoses). Also note that when a Z-code is reported in

- **Column 2 :** the code for the underlying condition can often be entered in Column 2, as long as it is an active on-going condition impacting home health care. Rate the degree of symptom control for the condition listed in Column 1. Do not assign a symptom control rating if the diagnosis code is a V, W, X, Y or Z-code. Choose one value that represents the degree of symptom control appropriate for each diagnosis using the following scale: 0 - Asymptomatic, no treatment needed at this time 1 - Symptoms well controlled with current therapy 2 - Symptoms controlled with difficulty, affecting daily functioning; patient needs ongoing monitoring 3 - Symptoms poorly controlled; patient needs frequent adjustment in treatment and dose monitoring 4 - Symptoms poorly controlled; history of re-hospitalizations Note that the rating for symptom control in Column 2 should not be used to determine the sequencing of the diagnoses listed in Column 1. These are separate items and sequencing may not coincide.

OASIS IS ALREADY EXPORTED, PRIMARY DIAGNOSIS AND OTHER DIAGNOSIS FROM 1 TO 5 ARE NOW DISABLED

PRIMARY DIAGNOSIS
G89.29 - Other chronic pain

Onset Date
02/26/2026

Severity
0 1 2 (3) 4 ^

Onset (Exacerbate)

OTHER DIAGNOSIS - 1
M43.8X9 - Other specified deforming dorsopathies, site unspecified

Onset Date
02/26/2026

Severity
0 1 2 (3) 4 ^

Onset (Exacerbate)

OTHER DIAGNOSIS - 2
M41.129 - Adolescent idiopathic scoliosis, site unspecified

Onset Date
02/26/2026

Severity
0 1 2 (3) 4 ^

Onset (Exacerbate)

OTHER DIAGNOSIS - 3
M54.50 - Low back pain, unspecified

Onset Date
02/26/2026

Severity
0 1 (2) 3 4 ^

Onset (Exacerbate)

OTHER DIAGNOSIS - 4
M79.7 - Fibromyalgia

Onset Date
02/26/2026

Severity
0 1 (2) 3 4 ^

Onset (Exacerbate)

OTHER DIAGNOSIS - 5
M19.90 - Unspecified osteoarthritis, unspecified site

Onset Date
02/26/2026

Severity
0 1 (2) 3 4 ^

Onset (Exacerbate)

M06.9 - Rheumatoid arthritis, unspecified

Onset Date
02/26/2029

Severity
0 1 (2) 3 4 ^

Onset (Exacerbate)

E11.9 - Type 2 diabetes mellitus without complications

Onset Date
02/26/2026

Severity
0 1 (2) 3 4 ^

Onset (Exacerbate)

I10. - Essential (primary) hypertension

Onset Date
02/26/2026

Severity
0 1 (2) 3 4 ^

Onset (Exacerbate)

I48.91 - Unspecified atrial fibrillation

Onset Date
02/26/2026

Severity

		○ 0 ○ 1 ◉ 2 ○ 3 ○ 4 ○ ^ ○ Onset ◉ Exacerbate
E03.9 - Hypothyroidism, unspecified	**Onset Date** **02/26/2026**	**Severity** ○ 0 ○ 1 ◉ 2 ○ 3 ○ 4 ○ ^ ○ Onset ◉ Exacerbate
K21.9 - Gastro-esophageal reflux disease without esophagitis	**Onset Date** **02/26/2026**	**Severity** ○ 0 ○ 1 ◉ 2 ○ 3 ○ 4 ○ ^ ○ Onset ◉ Exacerbate
G47.30 - Sleep apnea, unspecified	**Onset Date** **02/26/2026**	**Severity** ○ 0 ○ 1 ◉ 2 ○ 3 ○ 4 ○ ^ ○ Onset ◉ Exacerbate
F33.9 - Major depressive disorder, recurrent, unspecified	**Onset Date** **02/26/2026**	**Severity** ○ 0 ○ 1 ◉ 2 ○ 3 ○ 4 ○ ^ ○ Onset ◉ Exacerbate
Z91.81 - History of falling	**Onset Date** **02/26/2026**	**Severity** ○ 0 ○ 1 ○ 2 ○ 3 ○ 4 ◉ ^ ◉ Onset ○ Exacerbate
Z79.01 - Long term (current) use of anticoagulants	**Onset Date** **02/26/2026**	**Severity** ○ 0 ○ 1 ○ 2 ○ 3 ○ 4 ◉ ^ ◉ Onset ○ Exacerbate

(M1028) Active Diagnoses – Comorbidities and Co-existing Conditions – Check all that apply See OASIS Guidance Manual for a complete list of relevant ICD-10 codes.

- ☐ **1. Peripheral Vascular Disease (PVD) or Peripheral Arterial Disease (PAD)**
- ☐ **2. Diabetes Mellitus (DM)**
- ☑ **3. None of the above**

VITAL SIGNS & PAIN ASSESSMENT

(M1060) Height and Weight - While measuring, if the number is X.1-X.4 round down; X.5 or greater round up

A. Height (in inches). Record most recent height measure since the most recent SOC/ROC:
63

B. Weight (in pounds). Base weight on most recent measure in last 30 days; measure weight consistently, according to standard agency practice (for example, in a.m. after voiding, before meal, with shoes off, etc.:
130

BMI:
23.03

Comments:

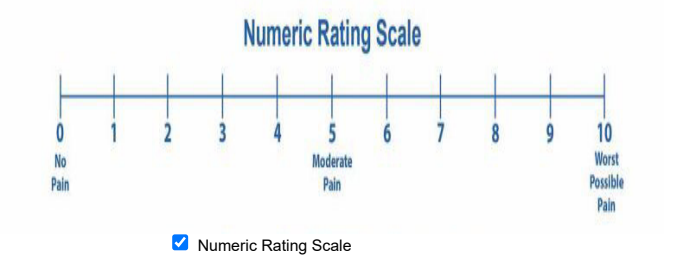
Vital Signs

Temperature: 97.3 Temporal
Respiratory (per minute): 18
Blood Pressure mm/Hg (R): 137 / 74 sitting
Blood Sugar (mg/dl): Non-Fasting
O2 Sat: 95
Comments:

Pulse (per minute): Apical = Radial = 87
Weight: 130 actual
Blood Pressure mm/Hg (L): /

O2 Sat in: Room Air

Pain Assessment



4

Collected Using: ☐ Face Scale ☑ Numeric Rating Scale

Pain Location and Description:

1

Location	Start Date	Activity	What makes pain worse?
Neck and back	02/26/2026	With Activity×	Activity×

Intensity	Duration	What makes pain better?	Does pain affect quality of life?
4×	at least a year	Med/Rest×	yes
	Unable to Assess ☐		Unable to Assess ☐

Description	Frequency	Type of Intervention
Throbbing×	Constant×	Non Opiod Analgesic×

How well it is working	Status
Fairly Well×	Treat×

Comments:
Patient reports chronic neck pain secondary to a MVA at the age of 17. She reports a history of four prior back surgeries with rod placement.

(J0510) Pain Effect on Sleep

Ask patient: "Over the past 5 days, how much of the time has pain made it hard for you to sleep at night"

- ○ 0. Does not apply – I have not had any pain or hurting in the past 5 days
- ○ 1. Rarely or not at all
- ○ 2. Occasionally
- ● 3. Frequently
- ○ 4. Almost constantly
- ○ 8. Unable to answer

(J0520) Pain Interference with Therapy Activities

Ask patient: "Over the past 5 days, how often have you limited your participation in rehabilitation therapy sessions due to pain?"

- ○ 0. Does not apply – I have not received rehabilitation therapy in the past 5 days
- ○ 1. Rarely or not at all
- ○ 2. Occasionally
- ● 3. Frequently
- ○ 4. Almost constantly
- ○ 8. Unable to answer

(J0530) Pain Interference with Day-to-Day Activities

Ask patient: "Over the past 5 days, how often you have limited your day-to-day activities (excluding rehabilitation therapy session) because of pain?"

- ○ 1. Rarely or not at all
- ○ 2. Occasionally
- ● 3. Frequently
- ○ 4. Almost Constantly
- ○ 8. Unable to answer

MEDICATION & ALLERGIES (INJECTABLE MEDICATIONS)

(N0415) High-Risk Drug Classes: Use and Indication:

	1. Is Taking 1. Is taking Check if the patient is taking any medications by pharmacological classification, not how it is used, in the following classes	2. Indication Noted 2. Indication noted If Column 1 is checked, check if there is an indication noted for all medications in the drug class
A. Antipsychotic	☐	☐
E. Anticoagulant	☑	☑
F. Antibiotic	☐	☐
H. Opioid	☐	☐
I. Antiplatelet	☐	☐
J. Hypoglycemic (including insulin)	☐	☐
Z. None of the Above	☐	

Medication

☐	Start Date	Medication	Strength / Dosage / Frequency	Route	Classification/Indication	Status
☐	07/28/2025	PREVACID	30 mg DR Cap Once a day	By mouth	GASTRIC MEDICATIONS,OTHER /	Old
☐	08/29/2025	ELIQUIS	5 mg 1 Tab Twice a day	By mouth	ANTICOAGULANTS / to reduce the risk of stroke and systemic embolism	New
☐	10/06/2025	CARDIZEM	120 mg 24 HR XR Tab Once a day	By mouth	CALCIUM CHANNEL BLOCKERS /	Old
☐	10/06/2025	MEDROL	4 mg 1 Tab Once a day	By mouth	GLUCOCORTICOIDS /	Old

	Start Date		Strength / Dosage / Frequency	Route	Classification/Indication	Status
☐	12/11/2025	CLINPRO 5000	1.1% Toothpaste Once a day	Toothpaste	CARIOSTATICS,TOPICAL /	Old
☐	01/20/2026	SYNTHROID	0.125 mg 1 Tab Once a day	By mouth	THYROID SUPPLEMENTS /	Old
☐	01/20/2026	Baclofen (Oral Pill)	10 mg 1 Tab Twice a day	By mouth	SKELETAL MUSCLE RELAXANTS /	Old
☐	01/20/2026	Carvedilol (Oral Pill)	25 mg 1 Tab Once a day	By mouth	BETA BLOCKERS/RELATED /	Old
☐	01/20/2026	LYRICA	150 mg Cap Once a day	By mouth	CNS MEDICATIONS,OTHER /	New
☐	02/20/2026	Terconazole (Vaginal)	80 mg Insert Once a day	Vaginal	ANTI-INFECTIVES,VAGINAL /	Old

Medication Remarks
Medication remarks...

☐ Discontinued Medications

Start Date	Medication	Strength / Dose / Frequency / Route	Classification/Indication	Laboratory	Status	Disc Date	Remarks

IV

Does patient have IV access? ○ **Yes** ● **No**

(M2001) Drug Regimen Review

Did a complete drug regimen review identify potential clinically significant medication issues?

- ● **0. No – No issues found during review _[Go to M2010, Patient/Caregiver High-Risk Drug Education]**
- ○ **1. Yes – Issues found during review**
- ○ **Not assessed/no information**
- ○ **9. NA – Patient is not taking any medications _[Go to O0110, Special Treatments, Procedures, and Programs]**

(M2003) Medication Follow-up

Did the agency contact a physician (or physician-designee) by midnight of the next calendar day and complete prescribed/recommended actions in response to the identified potential clinically significant medication issues?

- ○ **0. No**
- ○ **1. Yes**
- ○ **Not assessed/no information**

(M2010) Patient/Caregiver High-Risk Drug Education

Has the patient/caregiver received instruction on special precautions for all high-risk medications (such as hypoglycemics, anticoagulants, etc.) and how and when to report problems that may occur?

- ○ **0. No**
- ● **1. Yes**
- ○ **NA - Patient not taking any high-risk drugs OR patient/caregiver fully knowledgeable about special precautions associated with all high-risk medications**

(M2020) Management of Oral Medications

Patient's current ability to prepare and take all oral medications reliably and safely, including administration of the correct dosage at the appropriate times/intervals. Excludes injectable and IV medications. (NOTE: This refers to ability, not compliance or willingness.)

- ● **0. Able to independently take the correct oral medication(s) and proper dosage(s) at the correct times.**
- ○ **1. Able to take medication(s) at the correct times if: (a) individual dosages are prepared in advance by another person; OR (b) another person develops a drug diary or chart.**
- ○ **2. Able to take medication(s) at the correct times if given reminders by another person at the appropriate times.**
- ○ **3. Unable to take medication unless administered by another person**
- ○ **NA - No oral medications prescribed**

(M2030) Management of Injectable Medications

Patient's current ability to prepare and take all prescribed injectable medications reliably and safely, including administration of correct dosage at the appropriate times/intervals. Excludes IV medications.

- ○ **0. Able to independently take the correct medication(s) and proper dosage(s) at the correct times**
- ○ **1. Able to take injectable medication(s) at the correct times if: a. individual syringes are prepared in advance by another person; OR b. another person develops a drug diary or chart.**
- ○ **2. Able to take medication(s) at the correct times if given reminders by another person based on the frequency of the injection**
- ○ **3. Unable to take injectable medication unless administered by another person**
- ● **NA - No injectable medications prescribed.**

(O0110) Special Treatments, Procedures, and Programs

Check all of the following treatments, procedures, and programs that apply at discharge.

	On Admission (Check All That Apply)
Cancer Treatment	
A1. Chemotherapy	☐
A2. IV	☐
A3. Oral	☐
A10. Other	☐
B1. Radiation	☐
Respiratory Therapies	
C1. Oxygen Therapy	☐
C2. Continuous	☐
C3. Intermittent	☐
C4. High-Concentration	☐

D1. Suctioning	☐
D2. Scheduled	☐
D3. As needed	☐
E1. Tracheostomy Care	☐
F1. Invasive Mechanical Ventilator	☐
G1. Non-invasive Mech. Vent.	☐
G2. BiPAP	☐
G3. CPAP	☐
Others	
H1. IV Medications	☐
H2. Vasoactive medications	☐
H3. Antibiotics	☐
H4. Anticoagulation	☐
H10. Other	☐
I1. Transfusions	☐
J1. Dialysis	☐
J2. Hemodialysis	☐
J3. Peritoneal dialysis	☐
O1. IV Access	☐
O2. Peripheral	☐
O3. Mid-line	☐
O4. Central (e.g., PICC, tunneled, port)	☐
Z1. None of the Above	☑

Most Recent Immunizations

Pneumonia
○ Yes ● No ○ Unknown **Date:** mm/dd/yyyy

Flu
○ Yes ● No ○ Unknown **Date:** mm/dd/yyyy

Tetanus
○ Yes ○ No ● Unknown **Date:** mm/dd/yyyy

TB
○ Yes ○ No ● Unknown **Date:** mm/dd/yyyy

TB Exposure
○ Yes ● No ○ Unknown **Date:** mm/dd/yyyy

Hepatitis B
○ Yes ○ No ● Unknown **Date:** mm/dd/yyyy

Additional Immunizations

Immunization Name:
○ Yes ● No ○ Unknown **Date:** mm/dd/yyyy

Immunization Name:
○ Yes ● No ○ Unknown **Date:** mm/dd/yyyy

Comments:

Vaccination Record

1st Dose COVID-19
Product Name/Manufacturer (Lot No.): **Date:** mm/dd/yyyy **Healthcare Professional or Clinic Site:**

2nd Dose COVID-19
Product Name/Manufacturer (Lot No.): **Date:** mm/dd/yyyy **Healthcare Professional or Clinic Site:**

Booster 1
Product Name/Manufacturer (Lot No.): **Date:** **Healthcare Professional or Clinic Site:**

mm/dd/yyyy

Booster 2
Product Name/Manufacturer (Lot No.):

Date: mm/dd/yyyy

Healthcare Professional or Clinic Site:

Allergies

Add Allergies

Name	Reaction	Start Date
Compazine	Agitation	10/06/2025

NEUROLOGICAL (HEAD, MOOD, EYES, EARS)

Neuro/Emotional/Behavioral Assessment

Conditions of Participation
Neuromascular
Impairment:

- [ ] No Problems identified
- [ ] Speech
- [x] Hearing
- [ ] Visual
- [ ] Tremors
- [ ] Numbness
- [ ] Tingling
- [ ] Vertigo
- [ ] Ataxia
- [ ] Decreased Sensitivity

Others (Specify):

Psychosocial

- [x] WNL (Within Normal Limits)
- [ ] Poor Home Environment
- [ ] Impaired Decision Making
- [ ] Poor Coping Skills
- [ ] Demonstrated/Expressed Anxiety
- [ ] Agitated
- [ ] Inappropriate Behavior
- [ ] Depressed Mood
- [ ] Irritability
- [ ] Document as Clinical Narrative

Educational level
College bsn

Primary Language
English

Unable to: [ ] *Read* [ ] *Write*
Barriers:

Language
- ◯ Needs interpreter

Learning
- ◯ Mental
- ◯ Physical
- ◯ Psychosocial
- ◯ Functional

Emotions Reported:
- ◯ Angry
- ◯ Discouraged
- ◯ Withdrawn
- ◯ Flat affect
- ◯ Difficulty coping
- ◯ Disorganized

Treatment:
- ◯ Depressed
- ◯ Recent
- ◯ Long Term

Evidence of abuse/neglect/exploitation:
- ◯ Potential
- ◯ Verbal/Emotional
- ◯ Physical
- ◯ Actual
- ◯ Financial
- ● No Evidence of Abuse/Neglect/Exploitation

Unable to cope with altered health status, patient:
- ◯ Lacks motivation
- ◯ Has unrealistic expectations
- ● Denies problems
- ◯ Unable to recognize problems

Sleep/Rest
- ● Adequate
- ◯ Inadequate

No. of hours slept per night: 6

Comments

Spiritual / Cultural implications that impact care

Spiritual resource

[ ] Inappropriate responses to caregivers/clinician

[ ] Inappropriate follow-through in past

Did patient drive a vehicle before admission? [ ] Yes [x] No [ ] NA

Did the patient have a job before admission? ◯ Yes ● No

If yes, do they want to drive post-discharge? [ ] Yes [ ] No

If yes, do they want to return to work post-discharge? ◯ Yes ◯ No

(B1300) Health Literacy

How often do you need to have someone help you when you read instructions, pamphlets, or other written material from your doctor or pharmacy?

- ◯ **0. Never**
- ● **1. Rarely**
- ◯ **2. Sometimes**
- ◯ **3. Often**
- ◯ **4. Always**
- ◯ **7. Patient declines to respond**
- ◯ **8. Patient unable to respond**

(C0100) Should Brief Interview for Mental Status (C0200-C0500) be Conducted?

Attempt to conduct interview with all patients. ◯ **No (patient is rarely/never understood)** ● **Yes** ◯ **Not assessed/no information**

Brief Interview for Mental Status (BIMS)

(C0200) Repetition of Three Words

Ask patient: **"I am going to say three words for you to remember. Please repeat the words after I have said all three. The words are: SOCK , BLUE , and BED . Now tell me the three words."**

Number of words repeated after first attempt ◯ **None** ◯ **One** ● **Two** ◯ **Three** ◯ **Not Assessed**

After the patient's first attempt, repeat the words using cues (**"sock, something to wear; blue, a color; bed, a piece of furniture"**). You may repeat the words up to two more times.

(C0300) Temporal Orientation (Orientation to year, month, and day)

Ask patient: **"Please tell me what year it is right now?"**

A) Able to report correct year

- ◯ **0. Missed by > 5 years or no answer**
- ◯ **1. Missed by 2-5 years**
- ◯ **2. Missed by 1 year**
- ● **3. Correct**
- ◯ **Not assessed/no information**

Ask patient: **"What month are we in right now?"**

B) Able to report correct month

- ◯ **0. Missed by > 1 month or no answer**
- ◯ **1. Missed by 6 days to 1 month**
- ● **2. Accurate within 5 days**
- ◯ **Not assessed/no information**

Ask patient: **"What day of the week is today?"**

C) Able to report correct day of the week

- ◯ **0. Incorrect or no answer**
- ● **1. Correct**
- ◯ **Not assessed/no information**

(C0400). Recall

Ask patient: **"Let's go back to an earlier question. What were those three words that I asked you to repeat?" If unable to remember a word, give cue (something to wear; a color; a piece of furniture) for that word."**

A) Able to recall "sock"

- ◯ **0. No – could not recall**
- ◯ **1. Yes, after cueing ("something to wear")**
- ● **2. Yes, no cue required**
- ◯ **Not assessed/no information**

B) Able to recall "blue"

- ◯ **0. No – could not recall**
- ◯ **1. Yes, after cueing ("a color")**
- ● **2. Yes, no cue required**
- ◯ **Not assessed/no information**

C) Able to recall "bed"

- ◯ **0. No – could not recall**
- ● **1. Yes, after cueing ("a piece of furniture")**
- ◯ **2. Yes, no cue required**
- ◯ **Not assessed/no information**

C0500. BIMS Summary Score

Add scores for questions C0200-C0400 and fill in total score (00-15) **Enter 99 if the patient was unable to complete the interview** **13**

Mental Status

- ● **1 - Oriented**
- ◯ **4 - Depressed**
- ◯ **7 - Agitated**
- ◯ **10 - Dementia**
- ◯ **13 - Wanders**
- ◯ **2 - Comatose**
- ◯ **5 - Disoriented**
- ● **8 - Alert**
- ◯ **11 - Assaultive**
- 14 - Other
- ◯ **3 - Forgetful**
- ◯ **6 - Lethargic**
- ◯ **9 - Confused**
- ◯ **12 - Delirious**

Describe the patient's mental status. Include general appearance, behaviors, emotional responses, mental functioning, overall social interaction as well as both clinical objective observations and subjective descriptions reported during this visit.

Patient was clean and appropriately dressed in clean clothing; she appeared well-groomed. Patient was tolerant, engaged, and pleasant. Patient was attentive and interacted appropriately throughout the encounter. Patient demonstrated intact cognitive functioning and appeared intellectually sharp.

Has there been a sudden/acute change in their mental status? ☐ Yes ☑ No Reported by: ☐ If yes, did the change coincide with something else like medication change, fall, or loss of a loved one? ☐ No ☐ Yes

Comments:

(C1310) Signs and Symptoms of Delirium (from CAM©)

A. Acute Onset of Mental Status Change

Is there evidence of an acute change in mental status from the patient's baseline?

● **0. No** ◯ **1. Yes**

B. Inattention

Did the patient have difficulty focusing attention, for example, being easily distractible or having difficulty keeping track of what was being said?

- ● **0. Behavior not present**
- ◯ **1. Behavior continuously present, does not fluctuate**
- ◯ **2. Behavior present, fluctuates (comes and goes, changes in severity)**
- ◯ **Not assessed/no information**

C. Disorganized thinking

Was the patient's thinking disorganized or incoherent (rambling or irrelevant conversation, unclear or illogical flow of ideas, or unpredictable switching from subject to subject)?

- ● **0. Behavior not present**
- ◯ **1. Behavior continuously present, does not fluctuate**
- ◯ **2. Behavior present, fluctuates (comes and goes, changes in severity)**
- ◯ **Not assessed/no information**

D. Altered level of consciousness

Did the patient have altered level of consciousness, as indicated by any of the following criteria?

- **vigilant** – startled easily to any sound or touch □ lethargic – repeatedly dozed off when being asked questions, but responded to voice or touch
- **stuporous** – very difficult to arouse and keep aroused for the interview
- **comatose** – could not be arouse

- ● **0. Behavior not present**
- ◯ **1. Behavior continuously present, does not fluctuate**
- ◯ **2. Behavior present, fluctuates (comes and goes, changes in severity)**
- ◯ **Not assessed/no information**

(D0150) Patient Mood Interview (PHQ-2 to 9)

Say to patient: **"Over the last 2 weeks, have you been bothered by any of the following problems?"**
If symptom is present, **enter 1 (yes) in column 1 , Symptom Presence** . If yes in column 1, then ask the patient: **"About how often have you been bothered by this?"**
Read and show the patient a card with the symptom frequency choices. Indicate response in column 2, Symptom Frequency

1) Symptom Presence

- **0. No (enter 0 in column 2)**
- **1. Yes (enter 0-3 in column 2)**
- **9. No response (leave column 2 blank)**

2) Symptom Frequency

- 0 - **Never or 1 day**
- 1 - **2-6 days (several days)**
- 2 - **7-11 days (half or more of the days)**
- 3 - **12-14 days (nearly every day)**

	Presence	Frequency
A. Little interest or pleasure in doing things	**No** ×	**Never or 1 day**
B. Feeling down, depressed, or hopeless	**Yes** ×	**2-6 days (several days)** ×

If either **D0150A2 or D0150B2** is coded **2 or 3** , **CONTINUE** asking the questions below. If not, **END the PHQ interview**

	Presence	Frequency
C. Trouble falling or staying asleep, or sleeping too much	^	^
D. Feeling tired or having little energy	^	^
E. Poor appetite or overeating	^	^
F. Feeling bad about yourself – or that you are a failure or have let yourself or your family down	^	^
G. Trouble concentrating on things, such as reading the newspaper or watching television	^	^
H. Moving or speaking so slowly that other people could have noticed. Or the opposite – being so fidgety or restless that you have been moving around a lot more than usual	^	^
I. Thoughts that you would be better off dead, or of hurting yourself in some way	^	^
D0160. Total Severity Score Add scores for all frequency responses in Column 2, Symptom Frequency. Total score must be between 02 and 27. Enter 99 if unable to complete interview (i.e., Symptom Frequency is blank for 3 or more required items)		1

(D0700) Social Isolation

How often do you feel lonely or isolated from those around you?

- ○ 0. Never
- ○ 1. Rarely
- ● 2. Sometimes
- ○ 3. Often
- ○ 4. Always
- ○ 7. Patient declines to respond
- ○ 8. Patient unable to respond

(M1740) Cognitive, behavioral, and psychiatric symptoms that are demonstrated at least once a week (Reported or Observed): (Mark all that apply.

- ☐ 1. Memory deficit: failure to recognize familiar persons/places, inability to recall events of past 24 hours, significant memory loss so that supervision is required
- ☐ 2. Impaired decision-making: failure to perform usual ADLs or IADLs, inability to appropriately stop activities, jeopardizes safety through actions
- ☐ 3. Verbal disruption: yelling, threatening, excessive profanity, sexual references, etc.
- ☐ 4. Physical aggression: aggressive or combative to self and others (for example, hits self, throws objects, punches, dangerous maneuvers with wheelchair or other objects)
- ☐ 5. Disruptive, infantile, or socially inappropriate behavior (excludes verbal actions)
- ☐ 6. Delusional, hallucinatory, or paranoid behavior
- ☑ 7. None of the above behaviors demonstrated

(M1745) Frequency of Disruptive Behavior Symptoms (Reported or Observed):

- ● 0. Never
- ○ 1. Less than once a month
- ○ 2. Once a month
- ○ 3. Several times each month
- ○ 4. Several times a week
- ○ 5. At least daily

(M1700) Cognitive Functioning

Patient's current (day of assessment) level of alertness, orientation, comprehension, concentration, and immediate memory for simple commands

- ● 0. Alert/oriented, able to focus and shift attention, comprehends and recalls task directions independently.
- ○ 1. Requires prompting (cuing, repetition, reminders) only under stressful or unfamiliar conditions.
- ○ 2. Requires assistance and some direction in specific situations (for example, on all tasks involving shifting of attention) or consistently requires low stimulus environment due to distractibility
- ○ 3. Requires considerable assistance in routine situations. Is not alert and oriented or is unable to shift attention and recall directions more than half the time.
- ○ 4. Totally dependent due to disturbances such as constant disorientation, coma, persistent vegetative state, or delirium.

(M1710) When Confused (Reported or Observed Within the Last 14 Days):

- ● 0. Never
- ○ 1. In new or complex situations only
- ○ 2. On awakening or at night only
- ○ 3. During the day and evening, but not constantly
- ○ 4. Constantly
- ○ 5. Patient nonresponsive

(M1720) When Anxious (Reported or Observed Within the Last 14 Days):

- ● 0. None of the time
- ○ 1. Less often than daily
- ○ 2. Daily, but not constantly
- ○ 3. All of the time
- ○ NA - Patient nonresponsive

Eyes and Ears

(B1000) Vision

Ability to see in adequate light (with glasses or other visual appliances)

- ● 0. Adequate – sees fine detail, such as regular print in newspapers/books
- ○ 1. Impaired – sees large print, but not regular print in newspapers/books
- ○ 2. Moderate impaired – limited vision; not able to see newspaper headlines but can identify objects
- ○ 3. Highly impaired – object identification in question, but eyes appear to follow objects
- ○ 4. Severely impaired – no vision or sees only light, colors or shapes; eyes do not appear to follow objects

Vision cont.

- ☐ No Problems Identified
- ☑ Glasses
- ☐ Macular Degeneration
- ☐ Blurred Vision
- ☐ Glaucoma
- ☐ Contact Lenses:
- ☐ Right
- ☐ Left
- ☐ Cataracts
- ☐ Blindness(Specify):
- ☐ Others (Specify):

Comments:
patient reports she uses readers

Pupils

- ☑ No Problems Identified
- ☐ PERRLA
- ☐ Anisocoria
- ☐ Right
- ☐ Left
- ☐ Others(Specify):

Comments:

(B0200) Hearing

Ability to hear (with hearing aid or hearing appliances if normally used)

- ○ 0. Adequate – no difficulty in normal conversation, social interaction, listening to TV

- [x] 1. Minimal difficulty – difficulty in some environments (e.g., when person speaks softly, or setting is noisy)
- [ ] 2. Moderate difficulty – speaker has to increase volume and speak distinctly
- [ ] 3. Highly impaired – absence of useful hearing

Hearing cont.

- [ ] No Problems Identified
- [x] Right Hearing Impairment
- [x] Left Hearing Impairment
- [ ] Deaf
- [ ] Speech Impairment (Specify):

Comments:

CARDIOPULMONARY (CHEST & THORAX)

Heart Sounds

- [x] Normal
- [x] Regular
- [ ] Irregular
- [ ] Murmur
- [ ] Gallops
- [ ] Clicks
- [ ] Friction Rule

Comments (If Abnormal (Explain)):

Edema

Upper:
- [ ] Right Extremities
- [ ] Left Extremities

Lower:
- [ ] Right Extremities
- [ ] Left Extremities

Others:

Comments:

.
- [ ] Cramps
- [ ] Claudication
- [ ] Fatigue

Capillary Refill:
- [x] > 3 secs
- [ ] < 3 secs

PT/INR: /

Peripheral Pulses

Radial:	[x] Strong/Bounding	[ ] Weak	[ ] Thready	[ ] Absent
Pedal:	[ ] Strong/Bounding	[x] Weak	[ ] Thready	[ ] Absent

Comments:

Chest/Thorax

Chest Pain

- [x] Denies
- [ ] Anginal
- [ ] Postural
- [ ] Localized
- [ ] Substernal
- [ ] Sharp
- [ ] Dull
- [ ] Aching
- [ ] Radiating
- [ ] Viselike
- [ ] Stabbing
- [ ] Others (Specify):

Associated with:
- [ ] Shortness of Breath
- [ ] Activity
- [ ] Rest
- [ ] Frequency/Duration:

Comments:

(M1400) When is the patient dyspneic or noticeably Short of Breath?

- [x] 0. Patient is not short of breath
- [ ] 1. When walking more than 20 feet, climbing stairs
- [ ] 2. With moderate exertion (for example, while dressing, using commode or bedpan, walking distances less than 20 feet)
- [ ] 3. With minimal exertion (for example, while eating, talking, or performing other ADLs) or with agitation
- [ ] 4. At rest (during day or night)

Respiratory Status

- [ ] Dyspnea:
- [ ] At Rest
- [ ] With Exertion/Activity
- [ ] Accessory Muscle Used
- [ ] Orthopnea
- [ ] Retraction
- [ ] Stridor

O2 lpm O2 Saturation %
- [ ] PRN
- [ ] Continuous

- [ ] Mechanical Ventilator:
- [ ] Intubated
- [ ] Tracheostomy

Others (Specify):

Comments:

Lung Sounds

Anterior:	Right: Clear	Left: Clear		
Posterior:	Upper Right: Clear	Upper Left: Clear	Lower Right: Diminished	Lower Left: Diminished

Comments:

Cough

- [x] None
- [ ] Dry
- [ ] Acute
- [ ] Chronic
- [ ] Non-Productive
- [ ] Productive:
- [ ] Not Assessed/No Information
- [ ] Yes
- [ ] No

Suction:
- [ ] Not Assessed/No Information
- [ ] Yes
- [ ] No

- [ ] Hemoptysis

Frequency: Amount:

Comments:

GASTROINTESTINAL & GENITOURINARY ASSESSMENT

Gastrointestinal (Mouth - Anus)

Mouth and Throat

- [x] No Problems Identified
- [ ] Dentures:
- [ ] Upper
- [ ] Lower
- [ ] Partial
- [ ] Bleeding Gums/Gingivitis
- [ ] Toothache
- [ ] Ulcerations
- [ ] Lesion
- [ ] Dysphagia
- [ ] Hoarseness
- [ ] Sore Throat
- [ ] Masses/Tumors

Comments:

Appetite

- [ ] Good
- [x] Fair
- [ ] Poor

Intake:
- [x] Adequate
- [ ] Inadequate

Abnormalities:

- [ ] Anoxeria
- [ ] Cachexia
- [ ] Nausea
- [ ] Vomiting
- [ ] Decreased Appetite
- [ ] Reflux/Indigestion
- [ ] Diarrhea
- [ ] Constipation
- [ ] Bowel Incontinence
- [ ] Other (Specify):

Comments:

(M1870) Feeding or Eating

Current ability to feed self meals and snacks safely. Note: This refers only to the process of ***eating***, ***chewing***, and ***swallowing***, not preparing the food to be eaten.

- (●) **0. Able to independently feed self.**
- ( ) **1. Able to feed self independently but requires: (a) meal set-up; OR (b) intermittent assistance or supervision from another person; OR (c) a liquid, pureed or ground meat diet.**
- ( ) **2. Unable to feed self and must be assisted or supervised throughout the meal/snack.**
- ( ) **3. Able to take in nutrients orally and receives supplemental nutrients though a nasogastric tube or gastrostomy.**
- ( ) **4. Unable to take in nutrients orally and is fed nutrients through a nasogastric tube or gastrostomy.**
- ( ) **5. Unable to take in nutrients orally or by tube feeding.**

Feeding Cont.

- [ ] **Formula**
- [ ] **Continuous Feeding** **cc:** **hours:**
- [ ] **Bolus:**
- [ ] **Gravity**
- [ ] **Pump** **cc:** **every hour(s):**

Flushing Protocol:

PEG/G-Tube Placement:

- [ ] **Placement Checked**
- [ ] **Residual Checked** **Amount:**

Comments:

(K0520) Nutritional Approaches

	On Admission (Check All That Apply)
A. Parenteral/IV feeding	[ ]
B. Feeding tube (e.g., nasogastric or abdominal (PEG))	[ ]
C. Mechanically altered diet – require change in texture of food or liquids (e.g., pureed food, thickened liquids)	[ ]
D. Therapeutic diet (e.g., low salt, diabetic, low cholesterol)	[x]
Z. None of the above	[ ]

#16 Nutritional Requirements

- [ ] NPO
- [ ] No Added Salt
- [x] Low Salt
- [x] Low Fat
- [ ] Low Cholesterol
- [x] No Concentrated Sweets
- [ ] Controlled Carbohydrates
- [ ] Regular
- [ ] Fluid Restriction:

Other:

Nutritional Health Screen	Score	Nutritional Health Screen	Score
( ) Without reason, has lost more than 10 lbs in the last 3 months	15	( ) Does not always have enough money to buy foods need	10
(●) Has an illness or condition that made pt change the type and/or amount of food eaten	10	( ) Eats alone most of the time	5
( ) Has open decubitus, ulcer, burn or wound	10		
( ) Eats fewer than 2 meals a day	10	(●) Takes 3 or more prescribed or OTC medications a day	5
( ) Has a tooth/mouth problem that makes it hard to eat	10	(●) Is not always physically able to cook and/or feed self and has no caregiver to assist	5
( ) Has 3 or more drinks of beer, liquor or wine almost every day	10	( ) Frequently has a diarrhea or constipation	5
		Total	20

(●) **Good Nutritional Status (Score 0 - 25)** ( ) **Moderate Nutritional Status (Score 25 - 54)** ( ) **High Nutritional Status (Score 55 - 100)**

Nutritional Status Commments:

Weight

- [x] **No Change**
- [ ] **Weight Gain**
- [ ] **Weight Loss**

Abdomen

- [x] **Abdominal Soft/Tender**
- [ ] **Abdominal Distention**
- [ ] **Girth(cm):**

Comments:

(M1620) Bowel Incontinence Frequency

- (●) **0. Very rarely or never has bowel incontinence**
- ( ) **1. Less than once weekly**
- ( ) **2. One to three times weekly**
- ( ) **3. Four to six times weekly**
- ( ) **4. On a daily basis**
- ( ) **5. More often than once daily**
- ( ) **NA - Patient has ostomy for bowel elimination**
- ( ) **UK - Unknown**

(M1630) Ostomy for Bowel Elimination

Does this patient have an ostomy for bowel elimination that (within the last 14 days): a) was related to an inpatient facility stay; or b) necessitated a change in medical or treatment regimen?

- (●) **0. Patient does not have an ostomy for bowel elimination**

1. Patient's ostomy was not related to an inpatient stay and did not necessitate change in medical or treatment regimen.
2. The ostomy was related to an inpatient stay or did necessitate change in medical or treatment regimen.

Ostomy/Stoma Appearance

Pinkished-Red | Moist | Abnormalities: | Purple | Black in Color | Dry
Gray | Bad Odor | Itches/Hurt | Other (Specify):

Bowel Sounds:

Abdominal Quadrants:

Right Lower	Right Upper	Left Lower	Left Upper	Last Bowel Movement:
Normoactive	Normoactive	Normoactive	Normoactive	02/25/2026

Comments:

Constipation

Chronic | Acute | Occasional | Internal Hemorrhoids
External Hemorrhoids | Laxative/Enema Use

Comments:

Stool Appearance

Within Normal Limits | Gray | Tarry | Black | Fresh Blood
Others (Specify):

Comments:

Genitourinary (Genitals & Urinary)

(M1600) Has this patient been treated for a Urinary Tract Infection in the past 14 days?

0. No
1. Yes
NA - Patient on prophylactic treatment
UK - Unknown

(M1610) Urinary Incontinence or Urinary Catheter Presence:

Patient is incontinent
- Day time
- Night time
- Day and Night
- Timed-voiding defers incontinence
- Occasional stress incontinence

Patient requires a urinary catheter (specifically: external, indwelling, intermittent, or suprapubic)
No incontinence or catheter (includes anuria or ostomy for urinary drainage)

Genitourinary Assessment

Within Normal Limits

Abnormalities:

Frequency | Dysuria | Urgency | Anuria | Nocturia | Oliguria
Retention | Hematuria | Irrigation | Incontinence | Diapers Used | Penile Discharge
Vaginal Bleeding/Discharge | Others (Specify):

Urine Color:

Clear | Transparent | Pale Yellow | Brownish Orange
Pinkish Red | Dark Yellow | Blue or Green | Foamy
Transparent Yellow

Urine Consistency:

Hematuria | Sediment | Odorous | Cloudy

Urinary Catheter:

External Catheter | Foley | Suprapubic | Nephrostomy

Last Catheter Change: mm/dd/yyyy | Fr: | cc:

Comments:

INTEGUMENTARY (SKIN & WOUND)

(M1306) Does this patient have at least one Unhealed Pressure Ulcer/Injury at Stage 2 or Higher or designated as Unstageable?
(Excludes Stage 1 pressure injuries and all healed pressure ulcers/injuries)

0. No **[→ Skip to M1322, Current Number of Stage 1 Pressure Injuries at SOC/ROC; Skip to M1324, Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable at DC]**
1. Yes

(M1311) Current Number of Unhealed Pressure Ulcers/Injuries at Each Stage

A1. Stage 2: Partial thickness loss of dermis presenting as a shallow open ulcer with a red or pink wound bed, without slough. May also present as an intact or open/ruptured blister. **Number of Stage 2 pressure ulcers**	^
B1. Stage 3: Full thickness tissue loss. Subcutaneous fat may be visible but bone, tendon, or muscle is not exposed. Slough may be present but does not obscure the depth of tissue loss. May include undermining and tunneling. **Number of Stage 3 pressure ulcers**	^
C1. Stage 4: Full thickness tissue loss with exposed bone, tendon, or muscle. Slough or eschar may be present on some parts of the wound bed. Often includes undermining and tunneling. **Number of Stage 4 pressure ulcers**	^
D1. Unstageable: Non-removable dressing/device: Known but not stageable due to non┐removable dressing/device **Number of unstageable pressure ulcers/injuries due to non-removable dressing/device**	^
E1. Unstageable: Slough and/or eschar: Known but not stageable due to coverage of wound bed by slough and/or eschar **Number of unstageable pressure ulcers due to coverage of wound bed by slough and/or eschar**	^
F1. Unstageable: Deep tissue injury Number of unstageable pressure injuries presenting as deep tissue injury	^

(M1322) Current Number of Stage 1 Pressure Injuries:
Intact skin with non-blanchable redness of a localized area usually over a bony prominence. Darkly pigmented skin may not have a visible blanching; in dark skin tones only it may appear with persistent blue or purple hues.
Stage 1 Pressure Injuries: **0**

(M1324) Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable:
(Excludes pressure ulcer/injury that cannot be staged due to a non-removable dressing/device, coverage of wound bed by slough and/or eschar, or deep tissue injury.)
Stage of Most Problematic: **Patient has no pressure ulcers or no stageable pressure ulcers**

(M1330) Does this patient have a Stasis Ulcer?

- [x] 0. No _[GO TO M1340]
- [ ] 1. Yes, patient has BOTH observable and unobservable stasis ulcers
- [ ] 2. Yes, patient has observable stasis ulcers ONLY
- [ ] 3. Yes, patient has unobservable stasis ulcers ONLY (known but not observable due to non-removable dressing/device) _[GO TO M1340]

(M1332) Current Number of Stasis Ulcer(s) that are Observable:

- [ ] 1. One
- [ ] 2. Two
- [ ] 3. Three
- [ ] 4. Four or more

(M1334) Status of Most Problematic Stasis Ulcer that is Observable

- [ ] 1. Fully granulating
- [ ] 2. Early/partial granulation
- [ ] 3. Not healing

(M1340) Does this patient have a Surgical Wound?

- [x] 0. No _[GO TO N0415]
- [ ] 1. Yes, patient has at least one observable surgical wound
- [ ] 2. Surgical wound known but not observable due to non-removable dressing/device _[GO TO N0415]

(M1342) Status of Most Problematic Surgical Wound that is Observable

- [ ] 0. Newly epithelialized
- [ ] 1. Fully granulating
- [ ] 2. Early/partial granulation
- [ ] 3. Not healing

Skin Color

- [x] Within Normal Limits
- [ ] Pale
- [ ] Jaundice
- [ ] Cyanotic

Skin Turgor:

- [x] Good
- [ ] Fair
- [ ] Poor

Skin Abnormalities:

- [x] Dry
- [x] Warm
- [ ] Cool
- [ ] Rash
- [ ] Ulcer
- [ ] Ostomy
- [ ] Incision
- [ ] Diaphoretic
- [ ] Others (Specify):

Additional Skin Comments

Norton Scale for Pressure Ulcer Risk Assessment

	Good (4)	Fair (3)	Poor (2)	Very Bad (1)
Physical Condition	○ Good	● Fair	○ Poor	○ Very Bad
Mental Condition	● Alert	○ Apathetic	○ Confused	○ Stuporous
Activity	○ Ambulant	● Walks With Help	○ Chairbound	○ Bedfast
Mobility	○ Full	● Slightly Impaired	○ Very Limited	○ Immobile
Incontinence	● None	○ Occassional	○ Usually Urinary	○ Urinary and Fecal
			Total Score:	17

Wound

Wound Worksheet

Fetch Wounds From Previous Wound Note

SAFETY & RISK ASSESSMENT

Home Safety Assessment

Home Environment

- [ ] Architectural Barriers
- [ ] No fire extinguisher
- [ ] No emergency exit plan
- [ ] Medications are improperly stored
- [ ] Inadequate electricity / cooling / heating
- [ ] Rodent / insect infestation
- [ ] No telephone
- [ ] Inadequate supply of water
- [ ] Unsafe electrical wiring
- [ ] Cluttered surroundings
- [ ] Inadequate sanitation / plumbing
- [ ] Nonworking stove / refrigerator
- [ ] Inadequate railing
- [ ] Inadequate smoke detectors on all levels of home
- [ ] Exposed flammables
- [ ] Unsafe oxygen use
- [ ] Unsafe storage of equipments
- [ ] Inadequate Lighting

Other:
Patient's home environment and safety was assessed during the visit and noted was safe for the patient.

Type of Assistance Patient Receives - other than from Home Health Agency Staff

	Family/Friends	Provider Services	Paid Caregiver	Volunteer Organizations
ADL(Bathing, dressing, toileting, eating/feeding, bowel)	[x]	[ ]	[ ]	[ ]
IADL (Meds, meals, house-keeping, laundry, telephone, shipping, finances)	[x]	[ ]	[ ]	[ ]
Psychosocial Support	[x]	[ ]	[ ]	[ ]
Assistance with doctors appointment, delivery of medications	[x]	[ ]	[ ]	[ ]
Management of Finances	[x]	[ ]	[ ]	[ ]

Comments:

(M2102) Types and Sources of Assistance

Determine the ability and willingness of non-agency caregivers (such as family members, friends, or privately paid caregivers) to provide assistance for the following activities, if assistance is needed. Excludes all care by your agency staff

F. Supervision and Safety (for example, due to cognitive impairment)

- ○ **0. No assistance needed – patient is independent or does not have needs in this area**
- ○ **1. Non-agency caregiver(s) currently provide assistance**
- ☑ **2. Non-agency caregiver(s) need training/supportive services to provide assistance**
- ○ **3. Non-agency caregiver(s) are not likely to provide assistance OR it is unclear if they will provide assistance**
- ○ **4. Assistance needed, but no non-agency caregiver(s) available**

Safety Hazards Affecting Patient

- ☐ **Stairs**
- ☐ **Inadequate lighting, heating and cooling**
- ☐ **Insect/Rodent Infestation**
- ☐ **No gas/electric appliance**
- ☐ **No running water, plumbing**
- ☐ **Narrow or obstructed walkway**
- ☐ **Lack of fire safety devices**
- ☐ **Cluttered/soiled living area**
- ☑ **No hazards identified**

Others:

Safety Measures

- ☑ **1 - Bleeding precautions**
- ○ **2 - O_2 precautions**
- ○ **3 - Seizure precautions**
- ☑ **4 - Fall precautions**
- ○ **5 - Aspiration precautions**
- ○ **6 - Siderails up**
- ○ **7 - Elevate head of bed**
- ○ **8 - 24 hr. supervision**
- ☑ **9- Clear Pathways up**
- ○ **10 - Lock w/c with transfers**
- ☑ **11 - Infection control measures**
- ☑ **12 - Walker/cane**
- ☑ **13 - Sharps Precaution**
- ☑ **14 - Other:**

ER/911 protocol; COVID-19 and universal precautions.

Hospitalization Risk Assessment

Prior Pattern: Check all that apply

- ☐ **> 1 Hospitalizations or ER visits in the past 12 months**
- ☑ **History of falls**

Chronic conditions: Check all that apply (M1020/1022/1024)

- ☐ **HF**
- ☑ **Diabetes**
- ☐ **COPD**
- ☐ **Chronic skin ulcers (Wound consult if indicated for any wounds)**
- ☐ **HIV/AIDS**

Risk factors: (Check all that apply)

- ☐ **Discharged from hospital or skilled nursing facility**
- ☑ **More than 2 secondary diagnoses**
- ☐ **Low socioeconomic status or financial concerns ♦**
- ☐ **Lives alone ►♦**
- ☐ **Inadequate support network ♦**
- ☑ **ADL assistance needed ►**
- ☐ **Home safety risks ► ♦**
- ☐ **Dyspnea ► ★**
- ☑ **Help with managing medications needed ► ★**
- ☐ **Non-compliance with medication regimen ♦ ★**
- ☐ **Confusion ♦ ★**
- ☐ **Pressure ulcer ★**
- ☐ **Stasis ulcer ★**
- ☐ **Overall Poor Status/Prognosis ■**
- ☐ **Low literacy level ♦**
- ☐ **Depression ♦**
- ☐ **►Consider Therapy referral (PT, OT, ST)**
- ☑ **♦Consider MSW referral**
- ☐ **■ Consider Hospice referral**
- ☐ **★Consider RN referral, if not ordered**

Total number of checked boxes is:

7

Your agency may want to select a threshold score to target patients at high risk.

(For example: 5 or greater risk factors may indicate that the patient is at risk for hospitalization. Note: This number is for convenience only and has not been tested or validated. The agency may modify the score based upon the needs of their patient population.)

Carry out patient specific interventions as appropriate/ordered, if patient is at risk for hospitalization: (Coordinate with M2250)

Referrals:

- ☑ **SN**
- ☑ **PT**
- ☐ **OT**
- ☐ **ST**
- ☐ **MSW**
- ☑ **HHA**
- ☐ **Dietary Consultant**

Other:

Consider implementing any of the following interventions, if patient is at risk for hospitalization:

- ☑ **Medication Management**
- ☑ **Medication Reconciliation**
- ☑ **Assess patient's: knowledge, ability, resources and adherence**
- ☑ **Education**
- ☑ **Patient/family education**
- ☐ **Enrollment into a disease management program**

(Specify):

- ☐ **Hospice/Palliative Referral**
- ☐ **Individualized Patient Emergency Care Plan**
- ☑ **Fall Prevention Program**
- ☐ **Phone Monitoring**
- ☐ **Telemonitoring**
- ☐ **Wound Care**
- ☐ **Front-loading Visits**
- ☑ **Care Coordination (physicians, hospitals, nursing homes ...)**

Immunizations (M1040, M1045, M1050, M1055)

- ☐ **Influenza**
- ☐ **Pneumococcal**

Fall Prevention Program:

Skilled nurse assessed patient/caregiver home environment, mobility, and functional limitations to identify potential fall risks and promote safety within home setting. SN reviewed patient ability to perform activities of daily living and discussed factors that may contribute to falls such as fatigue and muscle weakness, highlighting importance of pacing activities and maintaining a safe environment to prevent accidents. SN outlined home exercise program and walked through passive and active-assistive range of motion exercises with patient/caregiver to support joint mobility, maintain muscle strength, and improve endurance. Skilled nurse covered performing exercises within tolerance to help promote flexibility, posture, and overall functional ability.

Other:

(M1033) Risk for Hospitalization: Which of the following signs or symptoms characterize this patient as at risk for hospitalization? (Mark all that apply.)

- ☐ **1. History of falls (2 or more falls – or any fall with an injury – in the past 12 months)**
- ☐ **2. Unintentional weight loss of a total of 10 pounds or more in the past 12 months**
- ☐ **3. Multiple hospitalizations (2 or more) in the past 6 months**

- [ ] 4. Multiple emergency department visits (2 or more) in the past 6 months
- [ ] 5. Decline in mental, emotional, or behavioral status in the past 3 months
- [x] 6. Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months
- [x] 7. Currently taking 5 or more medications
- [ ] 8. Currently reports exhaustion
- [ ] 9. Other risk(s) not listed in 1 - 8
- [ ] 10. None of the above

Additional Risk Information:

Prior Hospitalization

Yes ○ **No** **Number of times:** 3

FALL RISK ASSESSMENT TOOL

MAHC 10* - FALL RISK ASSESSMENT TOOL

	Yes	No
Age 65+	✓	
Diagnosis (3 or more co-existing) Includes only documented medical diagnosis	✓	
Prior history of falls within 3 months An unintentional change in position resulting in coming to rest on the ground or at a lower level		✓
Incontinence Inability to make it to the bathroom or commode in timely manner. Includes frequency, urgency, and/or nocturia.		✓
Visual impairment Includes but not limited to, macular degeneration, diabetic retinopathies, visual field loss, age related changes, decline in visual acuity, accommodation, glare tolerance, depth perception, and night vision or not wearing prescribed glasses or having the correct prescription.		✓
Impaired functional mobility May include patients who need help with IADLS or ADLS or have gait or transfer problems, arthritis, pain, fear of falling, foot problems, impaired sensation, impaired coordination or improper use of assistive devices.	✓	
Environmental hazards May include but not limited to, poor illumination, equipment tubing, inappropriate footwear, pets, hard to reach items, floor surfaces that are uneven or cluttered, or outdoor entry and exits.		✓
Poly Pharmacy (4 or more prescriptions - any type) All PRESCRIPTIONS including prescriptions for OTC meds. Drugs highly associated with fall risk include but not limited to, sedatives, antidepressants, tranquilizers, narcotics, antihypertensives, cardiac meds, corticosteroids, anti-anxiety drugs, anticholinergic drugs, and hypoglycemic drugs.	✓	
Pain affecting level of function Pain often affects an individual's desire or ability to move or pain can be a factor in depression or compliance with safety recommendations.	✓	
Cognitive impairment Could include patients with dementia, Alzheimer's or stroke patients or patients who are confused, use poor judgment, have decreased comprehension, impulsivity, memory deficits. Consider patients ability to adhere to the plan of care.		✓
A score of 4 or more is considered at risk for falling	5	

Fire Assessment for Patient with Oxygen

- [x] **Patient is not using oxygen**

Does patient have No Smoking signs posted?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Does patient or anyone in the home smoke with oxygen in use?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Are smoke detectors present and working properly?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Does patient have a properly functioning fire extinguisher?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Are oxygen cylinders stored properly?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Are all electrical cords near oxygen intact and free for fraying?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Does patient have an evacuation plan in case of fire?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Are all cleaning fluids and aerosols stored away from oxygen, and not used when oxygen is in use?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Does patient refrain from using petroleum products around oxygen?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No
Does patient only used water-based body and lip moisturizers?	○ Yes	○ No
• **Are the Patient and Caregiver educated regarding this?**	○ Yes	○ No

Comments:

FUNCTIONAL ASSESSMENT (SELF CARE)

(GG0100) Prior Functioning: Everyday Activities: Indicate the patient's usual ability with everyday activities prior to the current illness, exacerbation, or injury.

- **3. Independent** – Patient completed the activities by him/herself, with or without an assistive device, with no assistance from a helper
- **2. Needed Some Help** – Patient needed partial assistance from another person to complete activities
- **1. Dependent** – A helper completed the activities for the patient.
- **8. Unknown**
- **9. Not Applicable**

	Independent	Needed Some Help	Dependent	Unknown	Not Applicable
A. Self Care: Code the patient's need for assistance with bathing, dressing, using the toilet, or eating prior to the current illness, exacerbation, or injur	✓				

B. Indoor Mobility (Ambulation): Code the patient's need for assistance with walking from room to room (with or without a device such as cane, crutch or walker) prior to the current illness, exacerbation, or injury.	✓				
C. Stairs: Code the patient's need for assistance with internal or external stairs (with or without a device such as cane, crutch, or walker) prior to the current illness, exacerbation or injury.					✓
D. Functional Cognition: Code the patient's need for assistance with planning regular tasks, such as shopping or remembering to take medication prior to the current illness, exacerbation, or injury.		✓			

(GG0110) Prior Device Use

Indicate devices and aids used by the patient prior to the current illness, exacerbation, or injury (**Check all that apply**)

- [ ] **A. Manual wheelchair**
- [ ] **B. Motorized wheelchair and/or scooter**
- [ ] **C. Mechanical lift**
- [x] **D. Walker**
- [ ] **E. Orthotics/Prosthetics**
- [ ] **Z. None of the above**

(GG0130) Self Care

(GG0130) SOC/ROC Performance

Code the patient's usual performance at SOC/ROC for each activity using the 6-point scale. If activity was not attempted at SOC/ROC, code the reason.
Safety and Quality of Performance – If helper assistance is required because patient's performance is unsafe or of poor quality, score according to amount of assistance provided.
Activities may be completed with or without assistive devices.

06. Independent – Patient completes the activity by him/herself with no assistance from a helper.
05. Setup or clean-up assistance – Helper sets up or cleans up; patient completes activity. Helper assists only prior to or following the activity.
04. Supervision or touching assistance – Helper provides verbal cues and/or touching/steadying and/or contact guard assistance as patient completes activity. Assistance may be provided throughout the activity or intermittently.
03. Partial/moderate assistance – Helper does LESS THAN HALF the effort. Helper lifts, holds or supports trunk or limbs, but provides less than half the effort.
02. Substantial/maximal assistance – Helper does MORE THAN HALF the effort. Helper lifts or holds trunk or limbs and provides more than half the effort.
01. Dependent – Helper does ALL of the effort. Patient does none of the effort to complete the activity. Or, the assistance of 2 or more helpers is required for the patient to complete the activity.

If activity was not attempted, code reason:
07. Patient refused
09. Not applicable – Not attempted and the patient did not perform this activity prior to the current illness, exacerbation or injury.
10. Not attempted due to environmental limitations (e.g., lack of equipment, weather constraints)
88. Not attempted due to medical conditions or safety concerns

	A. Eating	B. Oral Hygiene	C. Toileting Hygiene	E. Shower /bathe self	F. Upper body dressing	G. Lower body dressing	H. Putting on /taking off footwear
(06) Independent	✓						
(05) Setup or clean-up assistance							
(04) Supervision or touching assistance		✓	✓	✓	✓		
(03) Partial/moderate assistance						✓	✓
(02) Substantial/maximal assistance							
(01) Dependent							
(07) Patient refused							
(09) Not applicable							
(10) Not attempted due to environmental limitations							
(88) Not attempted due to medical conditions or safety concerns							
(-) Not assessed/no information							

(M1800) Grooming

Current ability to tend safely to personal hygiene needs (specifically: washing face and hands, hair care, shaving or make up, teeth or denture care, or fingernail care)

- [ ] **0. Able to groom self unaided, with or without the use of assistive devices or adapted methods.**
- [ ] **1. Grooming utensils must be placed within reach before able to complete grooming activities.**
- [x] **2. Someone must assist the patient to groom self.**
- [ ] **3. Patient depends entirely upon someone else for grooming needs**

(M1810) Ability to Dress Upper Body

Current **Ability to Dress Upper Body** safely (with or without dressing aids) including undergarments, pullovers, front-opening shirts and blouses, managing zippers, buttons, and snaps:

- [ ] **0. Able to get clothes out of closets and drawers, put them on and remove them from the upper body without assistance.**
- [ ] **1. Able to dress upper body without assistance if clothing is laid out or handed to the patient.**
- [x] **2. Someone must help the patient put on upper body clothing.**
- [ ] **3. Patient depends entirely upon another person to dress the upper body.**

(M1820) Ability to Dress Lower Body

Current **Ability to Dress Lower Body** safely (with or without dressing aids) including undergarments, slacks, socks or nylons, shoes:

- [ ] **0. Able to obtain, put on, and remove clothing and shoes without assistance.**
- [ ] **1. Able to dress lower body without assistance if clothing and shoes are laid out or handed to the patient.**
- [x] **2. Someone must help the patient put on undergarments, slacks, socks or nylons, and shoes.**
- [ ] **3. Patient depends entirely upon another person to dress lower body.**

(M1830) Bathing

Current ability to wash entire body safely. **Excludes grooming (washing face, washing hands, and shampooing hair).**

- [ ] **0. Able to bathe self in shower or tub independently, including getting in and out of tub/shower.**
- [ ] **1. With the use of devices, is able to bathe self in shower or tub independently, including getting in and out of the tub/shower.**
- [ ] **2. Able to bathe in shower or tub with the intermittent assistance of another person: (a) for intermittent supervision or encouragement or reminders, OR (b) to get in and out of the shower or tub, OR (c) for washing difficult to reach areas.**
- [x] **3. Able to participate in bathing self in shower or tub, but requires presence of another person throughout the bath for assistance or supervision.**
- [ ] **4. Unable to use the shower or tub, but able to bathe self independently with or without the use of devices at the sink, in chair, or on commode.**
- [ ] **5. Unable to use the shower or tub, but able to participate in bathing self in bed, at the sink, in bedside chair, or on commode, with the assistance or supervision of another person**
- [ ] **6. Unable to participate effectively in bathing and is bathed totally by another person.**

(M1840) Toilet Transferring

Current ability to get to and from the toilet or bedside commode safely and transfer on and off toilet/commode

- ◯ **0. Able to get to and from the toilet and transfer independently with or without a device**
- ✓ **1. When reminded, assisted, or supervised by another person, able to get to and from the toilet and transfer.**
- ◯ **2. Unable to get to and from the toilet but is able to use a bedside commode (with or without assistance).**
- ◯ **3. Unable to get to and from the toilet or bedside commode but is able to use a bedpan/urinal independently**
- ◯ **4. Is totally dependent in toileting.**

(M1845) Toileting Hygiene

Current ability to maintain perineal hygiene safely, adjust clothes and/or incontinence pads before and after using toilet, commode, bedpan, urinal. If managing ostomy, includes cleaning area around stoma, but not managing equipment.

- ◯ **0. Able to manage toileting hygiene and clothing management without assistance.**
- ◯ **1. Able to manage toileting hygiene and clothing management without assistance if supplies/implements are laid out for the patient.**
- ✓ **2. Someone must help the patient to maintain toileting hygiene and/or adjust clothing.**
- ◯ **3. Patient depends entirely upon another person to maintain toileting hygiene**

(M1850) Transferring

Current ability to move safely from bed to chair, or ability to turn and position self in bed if patient is bedfast

- ◯ **0. Able to independently transfer.**
- ✓ **1. Able to transfer with minimal human assistance or with use of an assistive device.**
- ◯ **2. Able to bear weight and pivot during the transfer process but unable to transfer self.**
- ◯ **3. Unable to transfer self and is unable to bear weight or pivot when transferred by another person**
- ◯ **4. Bedfast, unable to transfer but is able to turn and position self in bed.**
- ◯ **5. Bedfast, unable to transfer and is unable to turn and position self**

(M1860) Ambulation/Locomotion

Current ability to walk safely, once in a standing position, or use a wheelchair, once in a seated position, on a variety of surfaces.

- ◯ **0. Able to independently walk on even and uneven surfaces and negotiate stairs with or without railings (specifically: needs no human assistance or assistive device).**
- ◯ **1. With the use of a one-handed device (for example, cane, single crutch, hemi-walker), able to independently walk on even and uneven surfaces and negotiate stairs with or without railings.**
- ◯ **2. Requires use of a two-handed device (for example, walker or crutches) to walk alone on a level surface and/or requires human supervision or assistance to negotiate stairs or steps or uneven surfaces.**
- ✓ **3. Able to walk only with the supervision or assistance of another person at all times.**
- ◯ **4. Chairfast, unable to ambulate but is able to wheel self independently.**
- ◯ **5. Chairfast, unable to ambulate and is unable to wheel self**
- ◯ **6. Bedfast, unable to ambulate or be up in a chair.**

FUNCTIONAL ASSESSMENT (MOBILITY & MUSCULOSKELETAL)

(GG0170) Mobility

(GG0170A) A. Roll left and right: The ability to roll from lying on back to left and right side, and return to lying on back on the bed.

SOC/ROC Performance

- ◯ **06. Independent**
- ◯ **05. Setup or clean-up assistance**
- ✓ **04. Supervision or touching assistance**
- ◯ **03. Partial/moderate assistance**
- ◯ **02. Substantial/maximal assistance**
- ◯ **01. Dependent**
- ◯ **07. Patient refused**
- ◯ **09. Not applicable**
- ◯ **10. Not attempted due to environmental limitations**
- ◯ **88. Not attempted due to medical conditions or safety concerns**
- ◯ **(-) Not assessed/no information**

(GG0170B) B. Sit to lying: The ability to move from sitting on side of bed to lying flat on the bed.

SOC/ROC Performance

- ◯ **06. Independent**
- ◯ **05. Setup or clean-up assistance**
- ✓ **04. Supervision or touching assistance**
- ◯ **03. Partial/moderate assistance**
- ◯ **02. Substantial/maximal assistance**
- ◯ **01. Dependent**
- ◯ **07. Patient refused**
- ◯ **09. Not applicable**
- ◯ **10. Not attempted due to environmental limitations**
- ◯ **88. Not attempted due to medical conditions or safety concerns**
- ◯ **(-) Not assessed/no information**

(GG0170C) C. Lying to sitting on side of bed: The ability to move from lying on the back to sitting on the side of the bed with feet flat on the floor, and with no back support.

SOC/ROC Performance

- ◯ **06. Independent**
- ◯ **05. Setup or clean-up assistance**
- ✓ **04. Supervision or touching assistance**
- ◯ **03. Partial/moderate assistance**
- ◯ **02. Substantial/maximal assistance**
- ◯ **01. Dependent**
- ◯ **07. Patient refused**
- ◯ **09. Not applicable**
- ◯ **10. Not attempted due to environmental limitations**
- ◯ **88. Not attempted due to medical conditions or safety concerns**
- ◯ **(-) Not assessed/no information**

(GG0170D) D. Sit to stand: The ability to come to a standing position from sitting in a chair, wheelchair, or on the side of the bed.

SOC/ROC Performance

- ◯ **06. Independent**
- ◯ **05. Setup or clean-up assistance**
- ✓ **04. Supervision or touching assistance**
- ◯ **03. Partial/moderate assistance**
- ◯ **02. Substantial/maximal assistance**
- ◯ **01. Dependent**
- ◯ **07. Patient refused**
- ◯ **09. Not applicable**
- ◯ **10. Not attempted due to environmental limitations**
- ◯ **88. Not attempted due to medical conditions or safety concerns**
- ◯ **(-) Not assessed/no information**

(GG0170E) E. Chair/bed-to-chair transfer: The ability to transfer to and from a bed to a chair (or wheelchair).

SOC/ROC Performance

- ◯ **06. Independent**
- ◯ **05. Setup or clean-up assistance**
- ✓ **04. Supervision or touching assistance**
- ◯ **03. Partial/moderate assistance**
- ◯ **02. Substantial/maximal assistance**
- ◯ **01. Dependent**
- ◯ **07. Patient refused**
- ◯ **09. Not applicable**
- ◯ **10. Not attempted due to environmental limitations**
- ◯ **88. Not attempted due to medical conditions or safety concerns**
- ◯ **(-) Not assessed/no information**

(GG0170F) F. Toilet transfer: The ability to get on and off a toilet or commode.

SOC/ROC Performance

- ◯ **06. Independent**
- ◯ **05. Setup or clean-up assistance**
- ✓ **04. Supervision or touching assistance**
- ◯ **03. Partial/moderate assistance**
- ◯ **02. Substantial/maximal assistance**
- ◯ **01. Dependent**
- ◯ **07. Patient refused**
- ◯ **09. Not applicable**
- ◯ **10. Not attempted due to environmental limitations**
- ◯ **88. Not attempted due to medical conditions or safety concerns**
- ◯ **(-) Not assessed/no information**

(GG0170G) G. Car transfer: The ability to transfer in and out of a car or van on the passenger side. Does not include the ability to open/close door or fasten seat belt.

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170I) I. Walk 10 feet: Once standing, the ability to walk at least 10 feet in a room, corridor, or similar space. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170M, 1 step (curb)*

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170J) J. Walk 50 feet with two turns: Once standing, the ability to walk 50 feet and make two turns.

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170K) K. Walk 150 feet: Once standing, the ability to walk at least 150 feet in a corridor or similar space.

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170L) L. Walking 10 feet on uneven surfaces: The ability to walk 10 feet on uneven or sloping surfaces (indoor or outdoor), such as turf or gravel.

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170M) M. 1 step (curb): The ability to go up and down a curb and/or up and down one step. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.*

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ● 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170N) N. 4 steps: The ability to go up and down four steps with or without a rail. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.*

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170O) O. 12 steps: The ability to go up and down 12 steps with or without a rail.

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170P) P. Picking up object: The ability to bend/stoop from a standing position to pick up a small object, such as a spoon, from the floor.

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ● 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170Q) Q. Does patient use a wheelchair and/or scooter?

- ● 0. No _**[GOTO M1600]**
- ○ 1. Yes
- ○ Not assessed (no information)

(GG0170R) R. Wheel 50 feet with two turns: Once seated in wheelchair/scooter, the ability to wheel at least 50 feet and make two turns.

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170RR) RR1. Indicate the type of wheelchair or scooter used.

SOC/ROC Performance

- ○ 1. Manual
- ○ 2. Motorized

- ◯ Not assessed (no information)

(GG0170S) S. Wheel 150 feet: Once seated in wheelchair/scooter, the ability to wheel at least 150 feet in a corridor or similar space.

SOC/ROC Performance

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170SS) SS1. Indicate the type of wheelchair or scooter used.

SOC/ROC Performance

- ◯ 1. Manual
- ◯ 2. Motorized
- ◯ Not assessed (no information)

Gait and Mobility Abnormalities

- ☐ No Problems Identified
- ☑ Weakness
- ☐ Bed Bound
- ☑ Joint Stiffness
- ☐ Orthopedic Aftercare
- ☐ Weight Bearing Restriction
- ☐ Fracture:
- ☐ Amputation:
- ☐ Paresthesia:
- ☐ Foot Drop: ☐ Right ☐ Left
- ☑ Poor Balance
- ☑ Chair Bound
- ☐ Shuffling/Wide-based Gait
- ☑ High Risk for Falls
- ☐ Poor Conditioning
- ☐ Joints Pain:
- ☐ Atrophy:
- ☐ Contractures: Site

Fall(s) since last visit (Specify):
last fall was in nov 2025

Weakness (Specify):
Stiffness, moving cautiously to avoid pain

Change in ADL (Explain):
Requires assistance with ADLs.

Comments:

Range of Motion

- ☐ Limited Range of Motion: ☐ RUE ☐ RLE ☐ LUE ☐ LLE
- ☐ Quadriplegia ☐ Hemiplegia ☐ Paraplegia

Comments:

Grip Strength

- ☐ Normal ☑ Strong ☐ Weak ☐ Equal
- ☐ Unequal ☐ Bilateral ☐ Right ☐ Left

Others (Specify):

Grasp Strength

☐ Normal ☑ Strong ☐ Weak ☐ Equal ☐ Unequal

Comments:

DME and Supplies

DME Lookup

Durable Medical Equipment:
Cane, Walker

#18B ACTIVITIES PERMITTED

- ◯ 1- Complete bedrest
- ◯ 2 - Bedrest/BRP
- ● 3 - Up as tolerated
- ● 4 - Transfer bed/chair
- ● 5 - Exercises prescribed
- ◯ 6 - Partial weight bearing
- ◯ 7 - Independent in home
- ◯ 8 - Crutches
- ● 9 - Cane
- ◯ A - Wheelchair
- ● B - Walker
- ◯ C - No restrictions
- ● D - Other (specify)

Assistance with ADLs.

ENDOCRINE (DIABETIC MANAGEMENT)

Endocrine

Is patient diabetic?	● Yes	◯ No	
Insulin dependent?	◯ Yes	● No	**How Long?**
Is patient independently able to draw up correct dose of insulin?	◯ Yes	◯ No	◯ N/A
Is patient able to administer own insulin?	◯ Yes	◯ No	◯ N/A
Is patient taking oral hypoglycemic agent?	◯ Yes	● No	◯ N/A
Is patient independent with glucometer use?	● Yes	◯ No	◯ N/A
Is caregiver able to draw up and administer insulin?	◯ Yes	◯ No	◯ N/A
Is caregiver independent with glucometer use?	● Yes	◯ No	◯ N/A
Does patient or caregiver routinely perform inspection of the patient's lower extremities?	● Yes	◯ No	◯ N/A

Diabetic Management

- ☑ Diet ☐ Hypoglycemic ☑ Exercise ☐ Insulin

Insulin Administered By:

- ☑ N/A ☐ Caregivier ☐ Patient ☐ Skilled Nurse

S&S of Hyperglycemia

☐ Polyuria ☐ Polyphagia ☐ Polydipsia ☐ Blurred Vision ☐ Fatigue

Others (Specify):

S&S of Hypoglycemia

☐ Perspiration ☐ Dizziness ☐ Irritability ☐ Pale ☐ Shakiness

Others (Specify):

Blood glucose range: **Low to High (mg/dL):**

Comments:

PLAN OF CARE AND SKILLED INTERVENTIONS

Plan of Care Orders

Orders and Treatment (Specify Frequency, Duration and Purpose of Visits): §484.60 (2.iv)

SN Frequency: 1x/wk x 4 wks **MSW Frequency:** **PT Frequency: Evaluation and treatment** **OT Frequency:**

HHA Frequency: 1 daya week **ST Frequency** **Dietitian Frequency:**

MD SIGNATURE COVERS ALL PAGES

ADMIT PATIENT TO STAR HOME HEALTH SERVICES. ALL DIAGNOSIS AND MEDICATIONS VERIFIED WITH THE PRIMARY CARE PHYSICIAN.

A 78-year-old female patient admitted to home health services due to chronic pain with associated deforming dorsopathies, scoliosis, low back pain, fibromyalgia, and osteoarthritis contributing to persistent discomfort, impaired balance, limited standing and walking tolerance, decreased endurance, and increased fall risk. Patient also presents with functional limitations requiring assistance with grooming, dressing, toileting, and supervision during activities requiring standing or ambulation, difficulty with position changes and mobility tasks, reliance on walker for support, mild hearing difficulty, occasional feelings of loneliness, polypharmacy with five or more medications including anticoagulant therapy, and need for caregiver assistance and support due to limited ability to safely perform some activities of daily living.

Skilled nurse services ordered to provide comprehensive assessment, chronic pain management support, medication management and safety review related to polypharmacy and anticoagulant precautions, fall prevention strategies, reinforcement of safe mobility techniques, support with activities of daily living, and caregiver instruction to promote safe assistance and proper care within home environment. Physical therapy services ordered to address decreased strength, impaired balance, limited endurance, and mobility limitations related to chronic musculoskeletal conditions. PT to provide therapeutic exercises, balance and strengthening activities, gait and mobility training, transfer training, and functional mobility training to improve safety, reduce fall risk, and help patient achieve highest possible level of independence with daily activities in home setting.

Frequency and duration:
Skilled Nurse: Once a week for 4 weeks for skilled observation and assessment, medication review and reconciliation, assess level of pain, infection control measures, compliance with treatment regimen and therapeutic diet, home safety and disease process teaching.
Physical Therapy: Evaluation and treatment for therapeutic exercise, strengthening, mobility, gait, transfer and fall prevention techniques training.
Home Health Aide: Twice a week for 4 weeks, starting on week of 11/16/2025 for ADLs/personal care assistance.

Patient’s goal: “I want to get stronger and better; be able to perform activities of daily living independently.”

Teachings/Interventions

Skilled Intervention

Assessment/Instruction/Performance:
Patient Covid-19 screening completed prior to visit. No report of symptoms or known COVID-19 exposure per patient/family. Patient provided with mask for use during HH visit. Patient agreed to use mask during treatment. Clinician performed hand hygiene, donned mask, face shield and gloves and followed infection control and bag technique procedures during the visit. No COVID related concerns during the time of visit. Disinfection of equipment followed including dry time. Proper disposal of wastes and doffing were done upon completion of the visit.

Explained Home Health service to patient/family/caregiver(s). Explained consent to care, release of liability, and patient rights & responsibilities. Consent to care, a liability release, and consent to photograph were signed by the patient/responsible party. A comprehensive assessment of all body systems with OASIS elements evaluation was initiated. Reconciled facility discharge medications against routine home prescriptions without issue. Discussed advance directives, privacy policy, COVID-19 prevention, 911 protocols, basic home safety, and fall prevention principles with patient/family/caregiver(s). Discussed the plan of care draft with the patient and explained what to expect in the next visits and the remainder of the care episode. Also explained that the initial discussion on the management of other disease processes will be done on the next visit.

1. Patient health status and homebound reason:

Mary Williams is a 78-year-old female with a significant functional limitation due to progressive spinal deformity, resulting in severe lower back pain that restricts her activities of daily living (ADLs). She has a medical history that includes arthritis, atrial fibrillation, chronic pain, type 2 diabetes mellitus, osteoarthritis, osteopenia, and a history of multiple hospitalizations due to pneumonia. The patient also experiences significant psychosocial challenges, including a lack of assistance at home, as her husband does not help with household tasks, further compounding her difficulties in managing her health and home environment. Mary's current medical status is concerning, as she demonstrates severe sagittal imbalance and coronal shift, which not only exacerbates her back pain but also impairs her mobility and overall functionality. She has reported an inability to stand for long periods and struggles with basic tasks such as folding laundry, leading to increased frustration and emotional distress. Her recent hospitalizations and ongoing symptoms highlight the necessity for continuous skilled care, particularly in pain management and rehabilitation.

Given her complex clinical picture, home health care is medically necessary to provide skilled nursing interventions and physical therapy aimed at pain management, mobility enhancement, and support with ADLs. The care team will focus on optimizing her pain control through medication management and exploring potential interventional pain management options, as well as ensuring that she receives necessary support for her daily activities. Additionally, the team will assess her home safety and make recommendations for adaptive equipment or home modifications to enhance her functional capacity and quality of life.

In summary, Mary's admission to home health is justified due to her severe spinal deformity and associated functional limitations, the need for skilled nursing and therapy services, and her recent hospitalizations that indicate a significant medical need for support in her recovery and chronic illness management. This approach aligns with Medicare home health guidelines, ensuring that she receives the comprehensive care required to improve her health outcomes.

2. Skilled Interventions:

Skilled nurse discussed chronic pain with patient/caregiver and explained how long-term pain can affect daily activities, sleep, and overall comfort if not managed properly. SN reviewed common triggers that may worsen discomfort and highlighted importance of pacing activities, maintaining proper body positioning, and allowing rest periods to help prevent pain flare-ups. Skilled nurse explained use of prescribed pain medications as directed and covered general expectations regarding relief and possible side effects that may occur. SN also explored non-medication approaches that may help reduce discomfort, such as gentle stretching, relaxation techniques, and use of supportive positioning during rest or activity. Skilled nurse walked through balancing activity with rest and outlined consistent pain management strategies to help support comfort and improve ability to carry out daily routines.

Skilled nurse discussed specified deforming dorsopathies with patient/caregiver and explained that changes in spinal structure may affect posture, flexibility, and comfort during daily activities. SN reviewed importance of maintaining proper body alignment when sitting, standing, and lying down to help reduce strain on back and support spinal stability. Skilled nurse highlighted use of supportive seating, proper lifting techniques, and avoiding sudden twisting or bending movements that may worsen discomfort or place added stress on spine. SN also covered pacing activities throughout day and allowing rest periods to prevent overexertion of back muscles. Skilled nurse walked through simple posture awareness and gentle movement strategies that may help maintain flexibility and support overall spinal health during routine activities.

Skilled nurse discussed fall preventive measures with patient/caregiver and reviewed common situations that may increase risk for falls within home setting. SN highlighted importance of keeping walkways clear of clutter, loose rugs, and electrical cords to create safer movement areas. Skilled nurse explained value of adequate lighting in hallways, bathrooms, and near bed to improve visibility during movement, especially at night. SN walked through safe mobility habits such as rising slowly from sitting or lying positions, wearing properly fitted non-slip footwear, and using assistive devices when needed for stability. Skilled nurse also covered placing frequently used items within easy reach and using handrails or grab bars in key areas of home to support balance and reduce risk of accidents.

☑ **Response to Teaching/Interventions**

Verbalized Understanding	☑ **Patient (%):** 70%	☑ **Caregiver (%):** 75%
Return Demonstration	☑ **Patient (%):** 65%	☑ **Caregiver (%):** 75%
Require Further Teaching	☑ **Patient**	☑ **Caregiver**

Comments:
Patient verbalized 70% of understanding on the disease process taught during this visit. Patient able to teach back 6 out of 10 about chronic pain, specified deforming dorsopathies (site unspecified) and fall preventive measures. Patient also redemonstrated 65% of home safety and fall prevention measures with the use of assistive device as per patient still demonstrates weakness, poor endurance and slow ambulation/unsteady gait. Caregiver verbalized 75% of understanding and is available to assist patient for safety with ADLs/IADLs, medication and diet compliance for the patient.

Instructions during visit

☑ **Assess/Observe** ☐ **Injection Administration** ☐ **Catheter change** ☐ **IV Therapy**
☐ **Wound Care** ☑ **Teaching/Instructions**

Patient Strength

☑ **Motivated Learner** ☑ **Strong Support System** ☐ **Absence of Multiple Diagnosis**
☐ **Enhanced Socioeconomic Status**

Other:

Conclusions

☑ **Skilled Intervention Needed** ☑ **Skilled Instruction Needed** ☐ **No Skilled Service Needed**

Other:

Rehabilitation Potential

☐ **Rehabilitation potential is poor** ☐ **Rehabilitation potential is good**
☑ **Rehabilitation potential is fair** ☐ **Rehabilitation potential is excellent**

Others:

Conference With

☑ **MD** ☑ **SN** ☐ **PT** ☐ **OT**
☐ **ST** ☐ **MSW** ☐ **HHA** ☑ **DON/DOPCS**
☐ **Clinical Coordinator** ☑ **Clinical Manager**

Name:

Regarding:
SOC, POC, Visit frequencies and update of patient health condition

Physician Contacted Re:
SOC, POC, Visit frequencies and update of patient health condition

Order Changes:
None

Plan for Next Visit:
SN will continue to assess the patient's overall health condition, patient/caregiver understanding of the disease process, functional status, and knowledge of self-care management by the next visit.

Next Physician Visit:

mm/dd/yyyy

Patient and caregiver education and training to facilitate timely discharge: §484.60 (2.xiii)
Initiated instruction and training provided by the SN, as appropriate, regarding the care and services identified in the plan of care.

Discharge Plans

Discharge when patient is independent in management of medical needs.
Discharge when caregiver willing and able to manage all aspects of patient's care.
Discharge to care of physician.
Discharge when reliable caregiver available to assist with patient's medical needs.
When patient knows when to notify physician.
Able to stay in residence with primary caregiver's assistance or community agency's support.
When goals are met.
Able to return to an independent level of care.
Discharge to caregiver.
Discharge patient to self-care.
Discharge when medical condition is stable and patient is no longer in need of skilled services.
Able to understand medication regime and care related to diagnoses.

☑ **Written notice of discharge provided to patient**

Discharge scheduled for:
mm/dd/yyyy

Discharge Plan Discussed with:

☑ **Patient/Family** ☑ **Care Manager** ☑ **Physician** ☐ **Other (specify):**

Discharge Plan:

☑ **To self-care when goals met** ☑ **To self care when max potential achieved** ☑ **To outpatient therapy with MD approval** ☐ **Others (Specify):**

Comments:

PATIENT SUMMARY & CLINICAL NARRATIVE

Summary

Mary Williams is a 78-year-old female patient who has been referred for home health care services on February 17, 2026, due to her primary diagnosis of G89.29, classified as Other chronic pain. The attending physician overseeing her care is Dr. William Stevens. The assessment for the start of care was completed by RN Toni Lara on February 26, 2026. It is noteworthy that Mary was not discharged from an inpatient facility within the past 14 days, indicating that her need for home health services is based solely on her chronic health issues and functional limitations.

Mary meets the criteria for homebound status due to her medical conditions. She experiences chronic pain and has deforming dorsopathies, making it medically contraindicated for her to leave her home. The patient's inability to tolerate standing or walking for extended periods, coupled with impaired balance, increases her risk for falls. Leaving her home is not only exhausting but also requires considerable effort, as it could exacerbate her condition and hinder her recovery. Therefore, she is classified as homebound, which is critical in determining her eligibility for home health services.

The primary reason for Mary requiring home health services stems from her chronic pain condition, which significantly affects her daily activities and overall quality of life. Alongside her primary diagnosis, she also has several comorbidities, including M43.8X9 (Other specified deforming dorsopathies), M41.129 (Adolescent idiopathic scoliosis), M54.50 (Low back pain, unspecified), M79.7 (Fibromyalgia), and M19.90 (Unspecified osteoarthritis). These conditions collectively contribute to her functional limitations and necessitate skilled nursing care.

A head-to-toe assessment reveals that Mary exhibits several health concerns. Cardiopulmonary assessments indicate a stable respiratory rate of 18 breaths per minute and a pulse of 87 beats per minute, with a blood pressure reading of 137/74 mmHg. Gastrointestinal assessments show no significant issues, while the integumentary system appears intact with no signs of pressure ulcers or wounds. Safe assessments indicate that Mary requires assistance with mobility, especially when transitioning from one position to another.

Psychosocially, Mary is currently experiencing minimal depression, as indicated by her PHQ-2 to 9 score of 1, which suggests she sometimes feels lonely or isolated. Her cognitive function is intact; she is alert, oriented, and able to follow directions independently, demonstrating no signs of confusion or disorganized thinking. Although she does have minimal difficulty with hearing in certain environments, her vision is adequate for reading standard print.

Mary's vital signs are stable, with a temperature of 97.3°F, indicating no signs of infection or acute distress. Her cognitive assessment reveals she is functioning well, with no evidence of altered consciousness or significant mental status changes. She is independent in her ability to eat but requires supervision for oral hygiene, toileting, and dressing, highlighting her need for assistance with several activities of daily living (ADLs).

In terms of functional ability, Mary is categorized as needing assistance with ADLs, such as grooming and dressing, where she requires help from another person. Despite this, she is independent in her indoor ambulation and can transfer with minimal assistance or the aid of a walker. Her mobility is limited, and she relies on a caregiver for supervision during tasks that require standing or walking.

Mary lives with other individuals who are not professional caregivers, indicating a need for training or supportive services to assist her adequately. Her social needs include a structured support system to help manage her health and assist with daily activities, as her current living arrangement does not provide sufficient care.

A thorough review of Mary's medications reveals she is currently taking five or more medications, which indicates a potential risk for drug interactions and noncompliance. Notably, she is on anticoagulants, which require special precautions and monitoring due to their high-risk nature. Instruction has been provided to both Mary and her caregivers regarding these medications to ensure proper adherence and to mitigate any adverse effects.

Mary's primary caregiver is a non-agency individual who may benefit from additional training in providing care. This caregiver plays a crucial role in assisting Mary with her daily needs and ensuring her safety at home. The support system around her must also be reinforced to ensure she receives adequate help and supervision during her recovery.

In conclusion, the medical necessity for skilled home health services is evident in Mary Williams' case. She requires consistent nursing assessments and interventions due to her chronic pain and comorbid conditions, which significantly impair her functional abilities and overall health. A visit frequency of at least two to three times per week is recommended to monitor her condition, provide necessary nursing care, and support her in achieving greater independence in her daily activities. This frequency will ensure that her health status is regularly evaluated and that any changes in her condition are promptly addressed.

CARE PLAN (PROBLEMS / GOALS / INTERVENTIONS)

Patient Care Plan / Goals / Interventions:

Problem: Osteoarthritis
Goals: Target Completion: 2 7 Term: Short-term Status: Unmet
Improve joint mobility and reduce pain.
Interventions: Assigned Staff: PT, SN
Instruct patient on joint protection techniques and safe mobility strategies.
Progress Toward Goals:
No Progress Yet
Assess pain levels and adjust pain management plan accordingly.
Progress Toward Goals:
No Progress Yet

Problem: Rheumatoid Arthritis
Goals: Target Completion: 2 7 Term: Short-term Status: Unmet
Manage symptoms and enhance quality of life.
Interventions: Assigned Staff: SN
Instruct patient on medication adherence and potential side effects.
Progress Toward Goals:
No Progress Yet
Assess joint function and evaluate the need for splinting.
Progress Toward Goals:

No Progress Yet
Implement low-impact aerobic exercises to improve endurance.
Progress Toward Goals:
No Progress Yet

Problem: Fall Risk
Goals: Target Completion: 4 7 Term: Long-term Status: Unmet
Reduce fall risk and enhance safety at home.
Interventions: Assigned Staff: OT, SN
Instruct on home safety assessments and modifications.
Progress Toward Goals:
No Progress Yet
Instruct family on emergency response measures in case of a fall.
Progress Toward Goals:
No Progress Yet
Encourage regular vision checks to address any visual impairments.
Progress Toward Goals:
No Progress Yet

Problem: Hypertension
Goals: Target Completion: 4 7 Term: Long-term Status: Unmet
Maintain blood pressure within normal limits.
Interventions: Assigned Staff: SN
Instruct on dietary changes, including sodium reduction.
Progress Toward Goals:
No Progress Yet
Assess blood pressure regularly to track management effectiveness.
Progress Toward Goals:
No Progress Yet
Instruct on stress management techniques to reduce hypertension triggers.
Progress Toward Goals:
No Progress Yet

Problem: Diabetes Mellitus (Type 1, Type 2)
Goals: Target Completion: 4 7 Term: Long-term Status: Unmet
Achieve and maintain optimal blood glucose levels.
Interventions: Assigned Staff: SN
Instruct on blood glucose monitoring and insulin administration.
Progress Toward Goals:
No Progress Yet
Implement a regular exercise program to support glucose control.
Progress Toward Goals:
No Progress Yet
Instruct on recognizing and managing hypoglycemia and hyperglycemia.
Progress Toward Goals:
No Progress Yet

Problem: Sleep Apnea
Goals: Target Completion: 2 7 Term: Short-term Status: Unmet
Improve sleep quality and reduce symptoms.
Interventions: Assigned Staff: SN
Instruct on the use and maintenance of CPAP devices.
Progress Toward Goals:
No Progress Yet
Assess sleep patterns and recommend sleep hygiene practices.
Progress Toward Goals:
No Progress Yet
Provide education on weight management to alleviate symptoms.
Progress Toward Goals:
No Progress Yet

Problem: Anxiety / Depression
Goals: Target Completion: 2 7 Term: Short-term Status: Unmet
Improve emotional well-being and coping strategies.
Interventions: Assigned Staff: SN
Instruct on stress-reduction techniques, such as breathing exercises.
Progress Toward Goals:
No Progress Yet
Assess mental health status and adjust care plan as necessary.
Progress Toward Goals:
No Progress Yet
Instruct family on supportive communication techniques.
Progress Toward Goals:
No Progress Yet

Problem: Chronic Pain
Goals: Target Completion: 4 7 Term: Long-term Status: Unmet
Manage pain effectively and enhance function.
Interventions: Assigned Staff: SN
Instruct on pain management strategies, including medication use.
Progress Toward Goals:
No Progress Yet
Assess pain levels regularly and adjust interventions as needed.
Progress Toward Goals:
No Progress Yet
Provide education on relaxation techniques to manage pain.
Progress Toward Goals:
No Progress Yet

Problem: Fibromyalgia
Goals: Target Completion: 2 7 Term: Short-term Status: Unmet
Manage symptoms and improve quality of life.
Interventions: Assigned Staff: SN
Instruct on lifestyle modifications to manage fatigue and pain.
Progress Toward Goals:
No Progress Yet
Instruct on stress management techniques to reduce symptom flare-ups.
Progress Toward Goals:
No Progress Yet
Encourage family participation in supportive activities.
Progress Toward Goals:
No Progress Yet

Problem: Hypothyroidism
Goals: Target Completion: 2 7 Term: Long-term Status: Unmet
Maintain normal thyroid function and manage symptoms.
Interventions: Assigned Staff: SN
Instruct on medication adherence and potential side effects.

Progress Toward Goals:
No Progress Yet
Assess symptoms regularly to evaluate treatment efficacy.
Progress Toward Goals:
No Progress Yet
Encourage family involvement in supporting medication adherence.
Progress Toward Goals:
No Progress Yet

Problem: Gastro-esophageal reflux disease without esophagitis
Goals: Target Completion: 2 7 Term: Short-term Status: Unmet
Manage symptoms and prevent complications.
Interventions: Assigned Staff: SN
Instruct on dietary modifications to reduce reflux symptoms.
Progress Toward Goals:
No Progress Yet
Assess symptom triggers and develop a management plan.
Progress Toward Goals:
No Progress Yet
Instruct on the importance of medication adherence.
Progress Toward Goals:
No Progress Yet

Ongoing Care Plans within OASIS date (02/26/2026):

Assigned Staff: Lara, Toni RN

Electronically Signed by Lara, Toni RN 02/26/2026